Australian Government
Australian Institute of Health and Welfare

Injury in Australia

Web report | Last updated: **18 Jun 2026** | Topic: Injury

About

Injury is a major cause of hospitalisation and death in Australia. This report presents annual updates of injury-related emergency department (ED) presentations, hospitalisations and deaths, including trends over the preceding decade.

Cat. no: INJCAT 213

- Summary
- Causes
- Data

Key findings

- Injuries result in around 5,000 ED presentations, 1,600 hospitalisations and 43 deaths each day in Australia
- Injuries represent about 20% of all ED presentations and 5% of all hospitalisations
- Injury hospitalisations and deaths are more likely in males, and people aged 65 and over
- Falls remain the leading cause of injury hospitalisations and deaths

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Summary

Injury is an Australia's health topic

In this section

- Introduction
- Key facts
- What are the leading causes of injury?
- Injury types
- Severity
- Variation in injuries by population groups
- Key data gaps and data improvement activities
- Where do I go for more information?



Injuries are a major cause of morbidity, permanent disability, and mortality, affecting the quality of life of individuals and families across Australia (AIHW 2008).

Injuries are the leading cause of death for people aged 1–44.

In 2024–25:

- males were more likely to present to an emergency department (ED), be hospitalised or die from injuries than females
- young children aged 0–4 had the highest rates of ED presentations due to injury of any age group
- people aged 65 and over were most likely to be hospitalised or die from an injury.

Data presented in this report are sourced from the National Non-admitted Patient Emergency Department Care Database (NNAPEDCD), the National Hospital Morbidity Database (NHMD) and the National Mortality Database (NMD).

Key facts

An overview of some key facts, using the latest available data is provided below.

ED presentations

1,824,454 ED presentations for injury (6,775 per 100,000 population) in 2023–24

The age-standardised rate of **ED presentations for injury** declined by an annual average of 1.9%, between 2018–19 and 2023–24.

Admitted patient care (Hospitals)

575,345 injury hospitalisations (2,136 per 100,000 population) in 2023–24

The age-standardised rate of **injury hospitalisations** remained stable between 2017–18 and 2023–24, falling very slightly by an annual average of 0.2%.

Impacts of injury

15,332 deaths (58 per 100,000 population) in 2022–23

The age-standardised rate of **injury deaths** remained stable between 2013–14 and 2022–23, rising very slightly by an annual average of 0.7%

7.9% of the total burden of disease in 2024 (AIHW 2024)

6.3% of health spending in 2023–24 (AIHW 2025)

What are the leading causes of injury?

Falls are the leading cause of injury hospitalisations and deaths in Australia.

The rate of fall-related injury hospitalisations has increased over the past decade, from 849 to 926 per 100,000 population between 2015–16 and 2024–25.

For more information on falls, see Injury in Australia: Falls.

After falls:

- the leading cause of injury hospitalisations in 2024–25 was contact with objects (such as knives, tools and machines) (300 per 100,000 population)
- the leading cause of injury deaths in 2023–24 was suicide (12 per 100,000 population).

The leading causes of injury hospitalisations and deaths vary by age and sex (Figure 1).

In 2024–25, the leading causes of injury hospitalisations were:

- contact with objects for males aged between 25 and 44 (18,600 hospitalisations)
- intentional self-harm for females aged between 15 and 24 (5,000 hospitalisations).

In 2023–24, the leading causes of injury deaths were:

- falls for people aged 65 and over (6,400 deaths); 95% of all deaths from falls occurred in this age group
- suicide for people aged between 15 and 64 (2,700 deaths, 82% of all deaths from suicide)
- transport-related injuries among people aged 0–14 (55 deaths, 3.7% of all transport deaths).

Figure 1: Leading causes of injury hospitalisation (2024–25) and death (2023–24), by age group and sex

Sex:
○ Females
● Males
○ Persons

	1st	2nd	3rd	4th	5th
0-4	Falls	Contact with objects	Thermal causes	Other unintentional causes	Contact with living things
5-14	Falls	Contact with objects	Transport	Contact with living things	Other unintentional causes
15-24	Transport	Contact with objects	Other unintentional causes	Falls	Contact with living things
25-44	Contact with objects	Other unintentional causes	Transport	Falls	Assault
45-64	Falls	Contact with objects	Transport	Other unintentional causes	Contact with living things
65+	Falls	Contact with objects	Transport	Other unintentional causes	Contact with living things
All ages	Falls	Contact with objects	Transport	Other unintentional causes	Contact with living things

https://www.aihw.gov.au

Rank chart by sex and age group for hospitalisations and age group only for deaths. Falls were the leading cause of hospitalisation and death for the 'All ages' category.

Notes

1. Data for age groups below 15 years of age are excluded for deaths due to small numbers.
2. 'All ages' includes hospitalisations or deaths where the age of the patient was not stated.
3. Person totals include hospitalisations where the sex of the patient was other, inadequately described or not stated.
4. 'Other unintentional causes' covers unintentional injury hospitalisations and deaths not principally caused by any of the main causes presented above, or where there may not have been enough information to specify the cause.

Sources: AIHW National Hospital Morbidity Database; AIHW National Mortality Database; ABS National, state and territory population.

Injury types

The most common injury types are fractures, open wounds and soft-tissue injuries (Figure 2). Fractures accounted for 407,000 ED presentations and 226,000 hospitalisations in 2024–25.

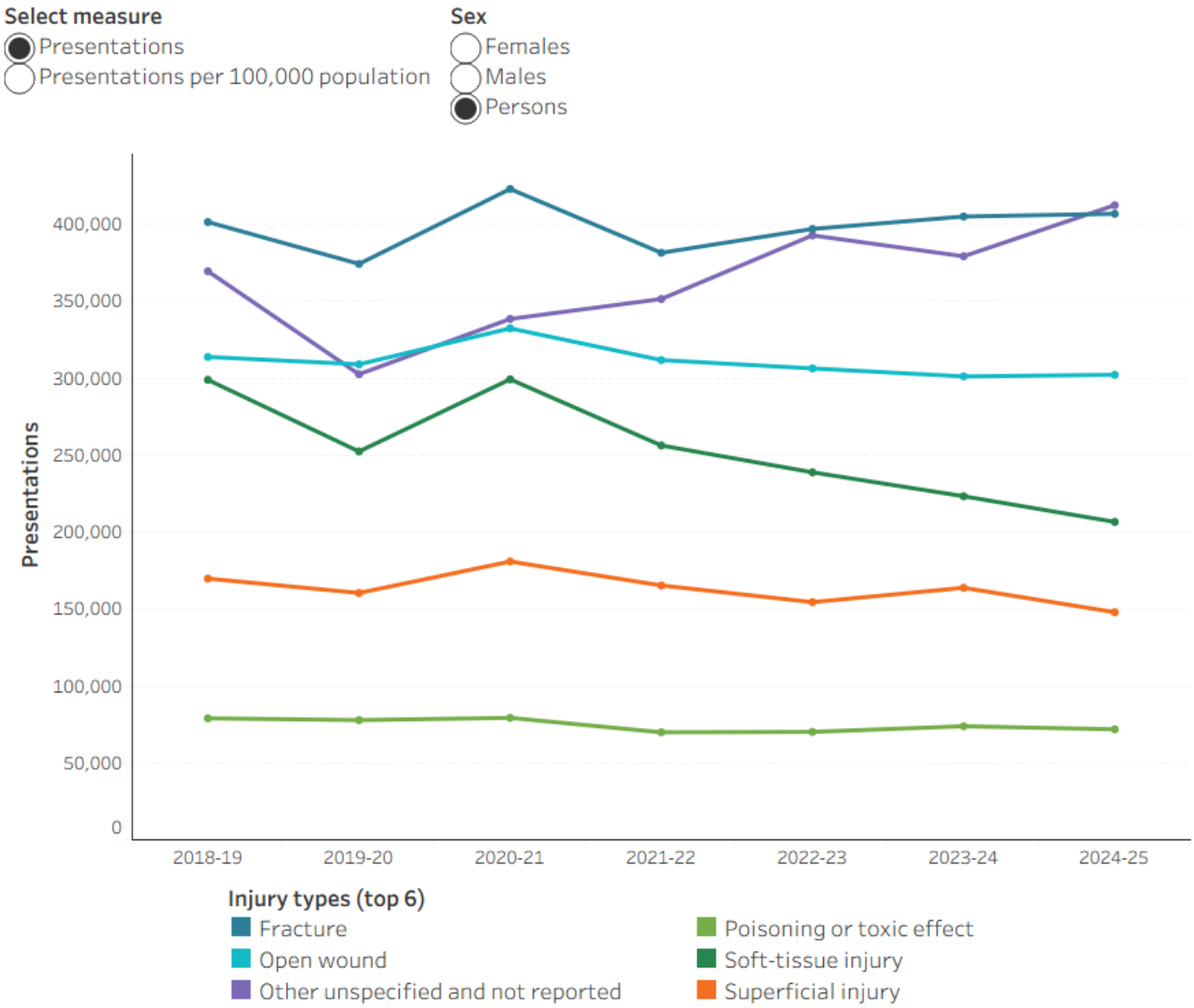
Injury types vary by age group. In 2024–25, older adults (65 and over) had the highest hospitalisation rates for:

- fractures (1,969 per 100,000 population)
- open wounds (652 per 100,000)
- superficial injuries (398 per 100,000)
- intracranial injuries (249 per 100,000).

People aged 65 and over were also more likely than other age groups to present to ED with dislocation and internal organ-related injuries.

Young children (0–4 years) were more likely to be hospitalised for burns and injuries involving foreign objects (through orifice) than other age groups. In ED presentations they showed higher rates for multiple injury types including burns, crushing injuries, dislocations, foreign object (through orifice) injuries, open wounds, and superficial injuries.

Figure 2: ED presentations, by type of injury and sex, 2018–19 to 2024–25



Notes

1. 'Persons' counts include cases where sex is intersex, indeterminate or missing. Therefore, the sum of components 'Males' and 'Females' may not equal the 'Persons' counts.
2. ED presentations are not reported by cause due to current data quality and poor capture of external cause data. See Injury in Australia technical notes for further detail.

Sources: AIHW National Non-admitted Patient Emergency Department Care Database; ABS National, state and territory population.

For more information, see Types of injury.

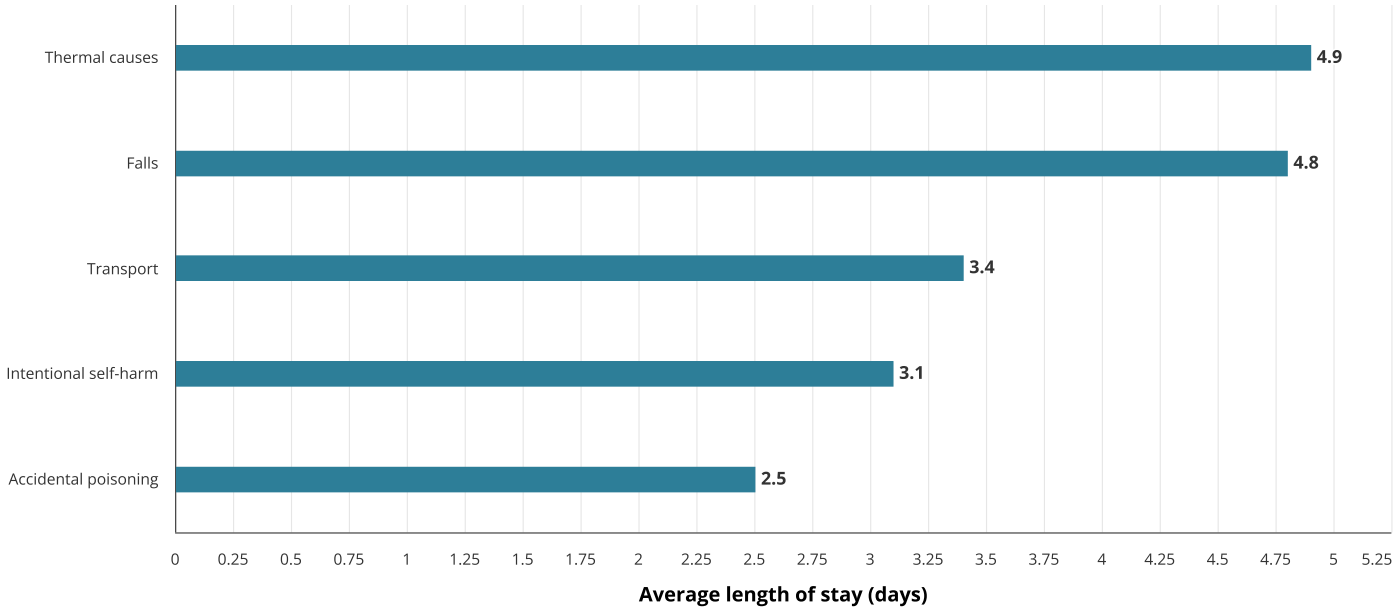
Severity

The severity or seriousness of an injury can be measured in several ways. Some of these include:

- ED presentation metrics such as triage category
- average length of stay in hospital measured in days
- time in an intensive care unit or receipt of continuous ventilatory support
- in-hospital deaths.

In 2024–25, the longest average hospital stays were for injuries caused by thermal sources and falls, with both averaging around 5 days (Figure 3).

Figure 3: Average length of stay in hospital by the top 5 injury causes, 2024–25



Notes

1. Average length of stay (ALOS) is the average number of days the group of patients stayed in hospital receiving treatment.
2. Length of stay calculations include transfer hospitalisations.

Source: AIHW National Hospital Morbidity Database.

For more information, see Severity.

Variation in injuries by population groups

Injuries can happen to anyone, but some population groups are more at risk than others.

First Nations people

Among Aboriginal and Torres Strait Islander (First Nations) people, there were:

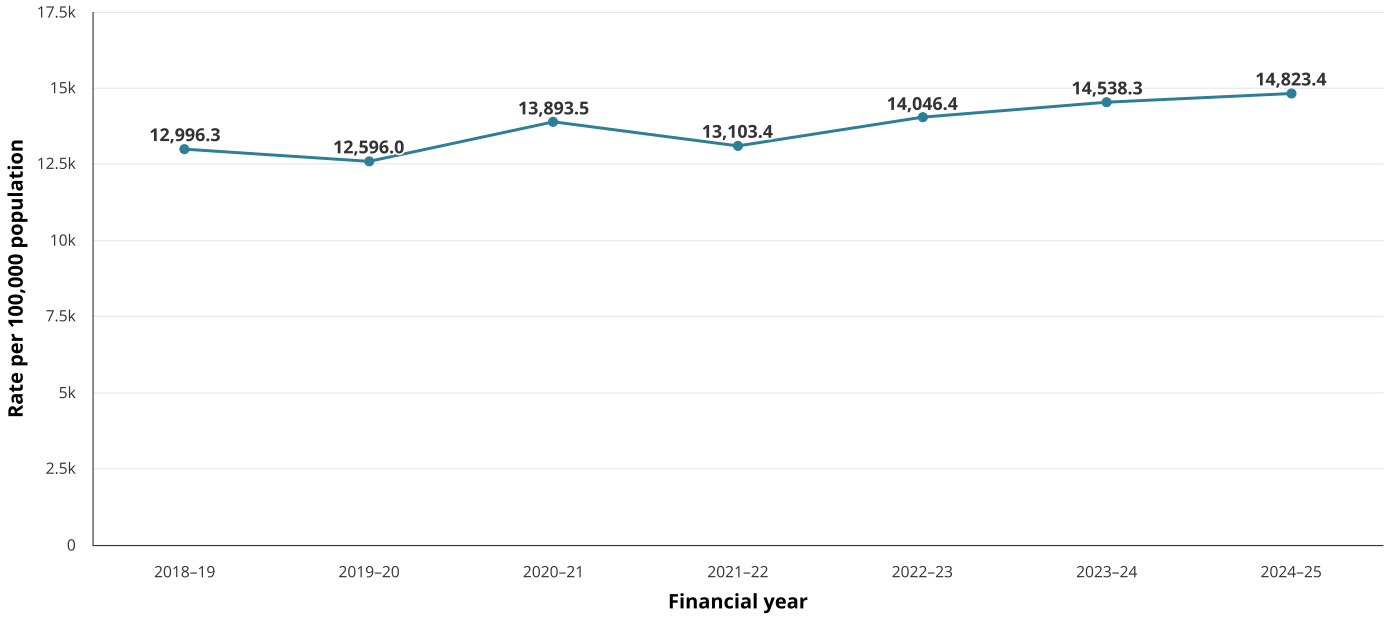
149,811 ED presentations for injury in 2023–24

35,775 injury hospitalisations in 2023–24

646 injury deaths in 2022–23

Between 2018–19 and 2024–25, ED presentations for injury have fluctuated between 13,000 and 15,000 per 100,000 population, with an overall average annual increase of 2.4%. More recently, the rate rose by 13%, from 13,103 in 2021–22 to 14,823 per 100,000 population in 2024–25.

Figure 4: Injury ED presentations for First Nations people, 2018–19 to 2024–25

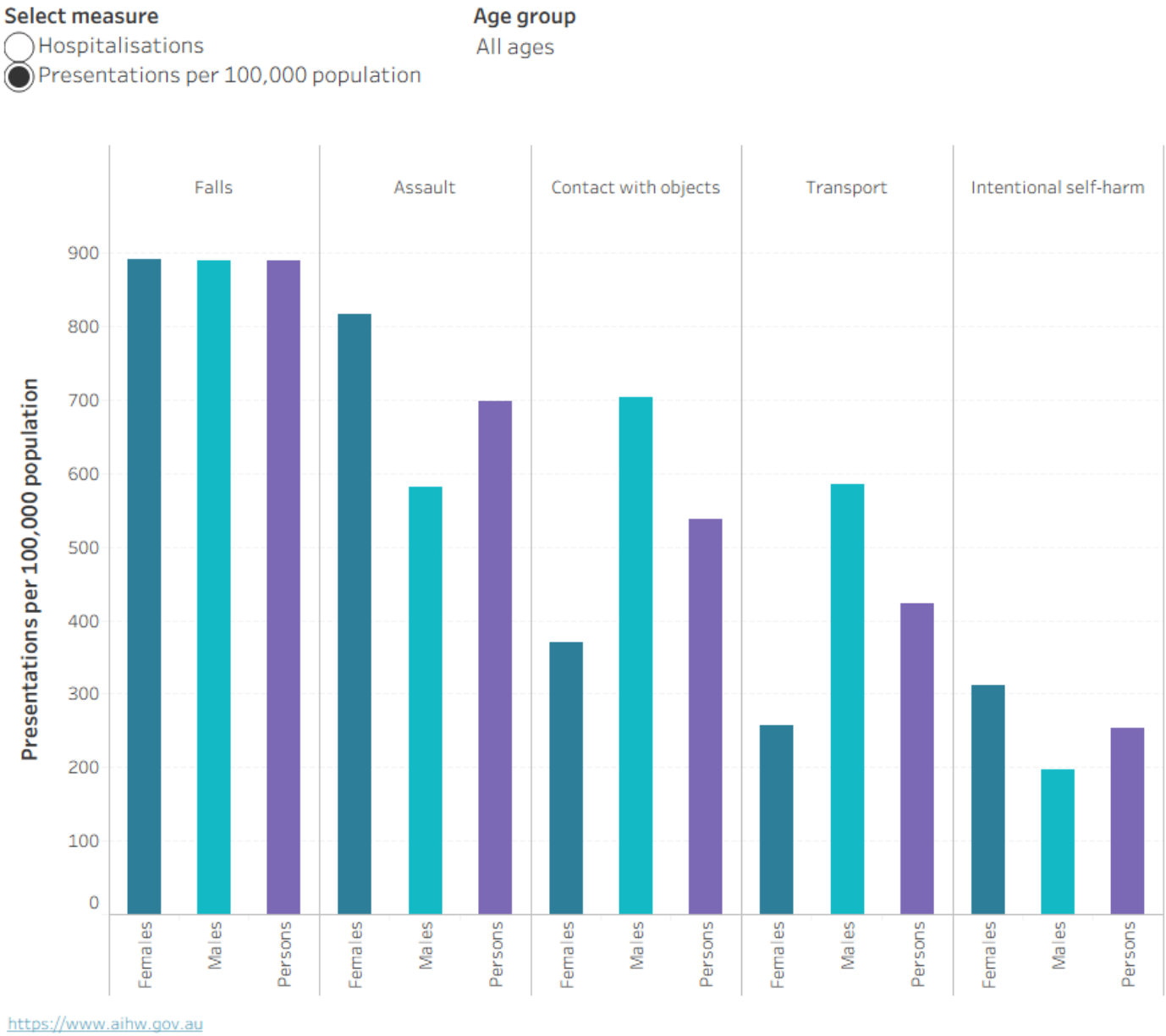


Note: Presentation rates are crude rates.
Source: AIHW National Non-admitted Patient Emergency Department Care Database; ABS Estimates and Projections, Aboriginal and Torres Strait Islander Australians

In 2024–25, falls were the leading cause of injury-related hospitalisations among First Nations people across all age groups, except for those aged 15–44 (Figure 5).

- Among females aged 15–44 and males aged 25–44, assault was the most common cause of hospitalisation.
- For males aged 15–24, the leading cause was transport accidents.

Figure 5: Leading causes of injury hospitalisation for First Nations people by age and sex, 2024-25



Notes

1. Data for intentional self-harm are aggregated for 0–14-year-olds.
2. 'All ages' includes hospitalisations where the age of the patient was not stated.
3. Person totals include hospitalisations where the sex of the patient was other, inadequately described or not stated.

Sources: AIHW National Hospital Morbidity Database; ABS Estimates and Projections, Aboriginal and Torres Strait Islander Australians.

For more information, see First Nations people.

Socioeconomic areas

Injuries are more common among people living in the most socioeconomically disadvantaged areas.

In 2023–24, compared to those in the least disadvantaged areas, individuals in the most disadvantaged areas were:

- 1.6 times as likely to present to an ED for injury
- 1.3 times as likely to be hospitalised for injury.

Falls were the leading cause of injury-related hospitalisations across all socioeconomic groups, with the age-standardised rate (ASR) highest among the most disadvantaged group (Figure 6).

This group also had the highest ASR for transport injuries, and assault ranked as the 5th most common cause of injury hospitalisation.

Figure 6: Age-standardised rates of the 5 most common injury hospitalisations by socioeconomic area, 2023–24



Sources: AIHW National Hospital Morbidity Database; ABS National, state and territory population.

For more information, see Socioeconomic areas.

Remoteness areas

The ASR of ED presentations and hospitalisations for injury increased with increasing remoteness.

In 2023–24, falls were the most common cause of injury-related hospitalisations across all remoteness areas – except in *Very remote* areas where assault was the leading cause (Figure 7).

Figure 7: Age-standardised rates of the 5 most common injury hospitalisations by remoteness area, 2023–24



Sources: AIHW National Hospital Morbidity Database; ABS National, state and territory population.

For more information, see Remoteness.

Key data gaps and data improvement activities

Gaps and limitations

The data presented in this report do not capture all injuries in Australia, such as those managed in primary care or by urgent care clinics, due to data availability.

The National Non-admitted Patient Emergency Department Care Database (NNAPEDCD) does not include external cause of injury data. External cause and related data on the injury include details such as place of occurrence, mechanism of the injury, activity being undertaken at the time, intent and perpetrator are only available in the NHMD.

Injury reporting in Australia currently relies on administrative health data, which limits its ability to identify the total number of individuals injured or track their full care pathways. Without linked data sources, it cannot account for multiple injuries or presentations across different health services, particularly affecting insights into priority populations such as people with disabilities and aged care residents.

Data developments and opportunities

National Injury Surveillance for Actionable Research – Emergency Department project (NISAR-ED)

The AIHW is partnering with the University of New South Wales in a Medical Research Future Fund funded initiative to build a National Injury Surveillance system, better identifying the external causes, intent and location of injuries treated in Emergency Departments at Australian hospitals.

Transport crash and injury data developments

The AIHW is working with the Department of Infrastructure's Office of Road Safety to investigate the feasibility of building a shared data asset of crash and injury data to support the National Road Safety Strategy (2021-2030) aims of reducing road trauma across Australia.

Primary health care data development

Nationally consistent primary health care data is a known information gap for effective population health monitoring, research, policy, and planning. The AIHW is working to fill this gap by developing processes for the governance, standardisation, collection, analysis and reporting of primary health care data within Australia. This work will ultimately form a National Primary Health Care Data Collection (NPHCDC). For more information, see Primary health care Data development.

Data linkage and integration

Data on the Australian health system are largely organised around occasions of service. Data linkage, also known as data integration, brings together information from more than one source. Matching disparate pieces of information can fill gaps in knowledge on not only specific diseases and the effectiveness and quality of health services and population groups but also across the health and welfare sectors.

The National Health Data Hub (NHDH) developed by the AIHW draws together core government administrative health, welfare, disability and aged care datasets. The NHDH is now an integral data resource in the Australian linked data landscape and contributes widely to an array of government programs.

The use of linked data will result in better information as to the nature, extent and consequences of injury, and its impact on the health system.

Where do I go for more information?

For more information on injury, see:

- Injury among women 2022–23
- Injuries affecting men in Australia: A closer look
- Injuries in children and adolescents 2021–22
- Extreme weather-related injuries in Australia over the last decade
- Spinal injuries in Australia 2020–21

For more on this topic, see Injury.

References

Australian Institute of Health and Welfare (AIHW) (2008) *Injury as a chronic health issue in Australia*, AIHW, Australian Government, accessed 22 April 2025.

AIHW (2024) *Australian Burden of Disease Study 2024*, AIHW, Australian Government, accessed 22 April 2025.

AIHW (2025) *Health system spending on disease and injury in Australia, 2023-24*, AIHW, Australian Government, accessed 29 October 2025.

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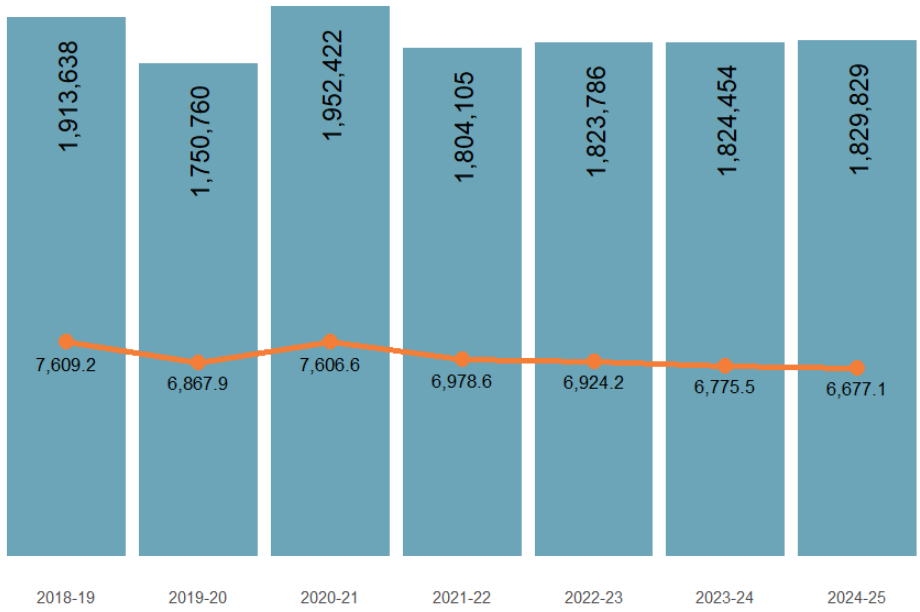
Injury trends

Emergency Department presentations

Between 2018–19 and 2024–25, rates of emergency department (ED) presentations for injury decreased by an annual average of 1.9%, with rates ranging from 7,609.2 in 2018–19 to 6,677.1 per 100,000 in 2024–25 (Figure 1). The presentation rate for injuries to EDs in 2024–25 was 5.0% lower than the previous 5-year average rate of 7,030.6 per 100,000 population.

The number of ED presentations for injury decreased over the same period. However, in the last 4 years, ED presentations for injury have increased, with about 25,000 more presentations in 2024–25 compared to 2021–22.

Figure 1: ED presentations for injuries, 2018–19 to 2024–25



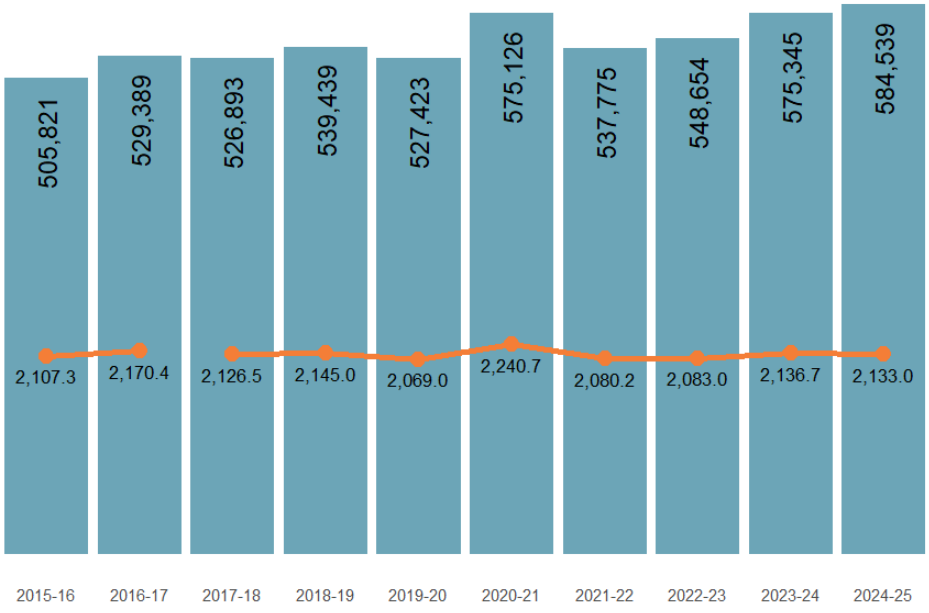
Note: Columns are case counts, while the line graph represents crude rates per 100,000 population.

Sources: AIHW National Non-admitted Patient Emergency Department Care Database and ABS National, state and territory population.

Injury hospitalisations

Over the past decade, hospitalisations from injury have remained relatively stable, with rates ranging from 2,069 to 2,240.7 per 100,000 persons in the last decade. During the same period, the number of injury hospitalisations rose from 505,821 to 584,539. The noticeable fluctuation in hospitalisation cases seen between 2020 and 2022 is likely due to disruptions caused by COVID-19 (Figure 2).

Figure 2: Injury hospitalisations by financial year, 2015–16 to 2024–25



Notes:

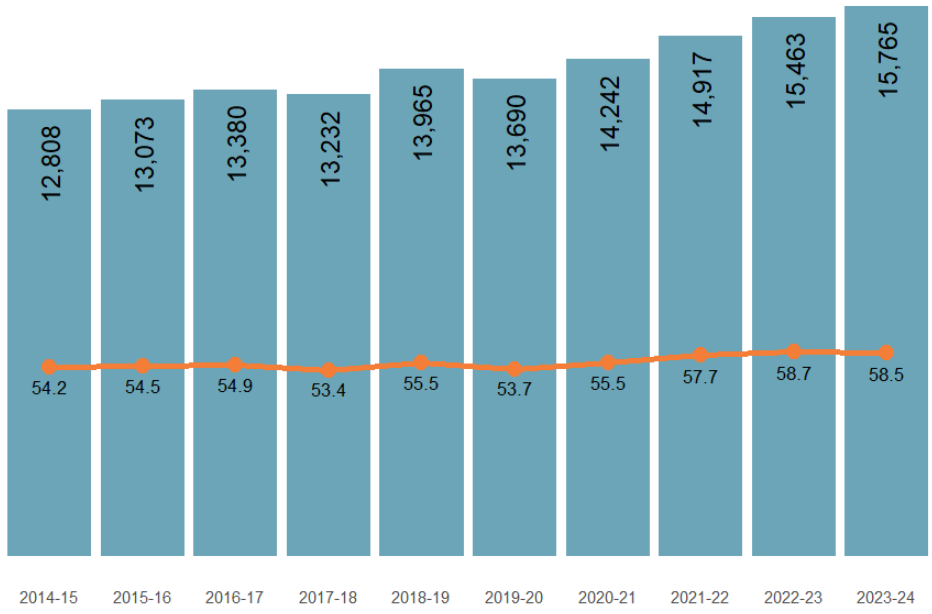
1. Columns are case counts, while the line graph represents crude rates per 100,000 population.
2. Break in series between 2016–17 and 2017–18. See technical notes for detail.

Sources: AIHW National Hospital Morbidity Database and ABS National, state and territory population.

Injury deaths

Over the past decade, deaths from injury have slightly increased, with rates ranging from 54.2 per 100,000 persons in 2014–15 to 58.5 in 2023–24, an average annual increase in the death rate of 0.9% (Figure 3). During the same period the number of injury deaths rose from 12,808 to 15,765.

Figure 3: Injury deaths by financial years, 2014–15 to 2023–24



Note: Columns are case counts, while the line graph represents crude rates per 100,000 population.

Sources: AIHW National Mortality Database and ABS National, state and territory population.

For more details, see Supplementary data tables E1, H3 and D3.

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Causes of injury

Injuries can be classified by cause once hospital staff or coroners discover how the person was injured. In some cases, they are not sure if it was accidental or deliberate, in which case the record will show *undetermined intent*.

The following articles each focus on a major cause of injury resulting in hospitalisation or death in Australia. See Ranked causes for an overview of injury hospitalisations and deaths by cause. ED presentations are not reported by cause due to the quality of the currently recorded information. Please refer to the technical notes for further details.

Unintentional causes



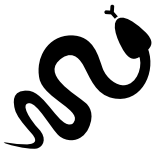
Falls



Contact with objects



Transport



Contact with people, plants and animals



Overexertion



Accidental poisoning



Thermal causes



Choking and suffocation



Electricity and air pressure



Forces of nature



Drowning and submersion



Other unintentional and undetermined intent

Intentional causes



Assault and homicide



Intentional self-harm and suicide

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Ranked causes

This chapter provides an overview of the main causes of injury hospitalisations (Figure 4) and deaths (Figure 5). Injuries have been categorised based on the first external cause of injury listed in each hospital and death record. ED presentations are not reported by cause due to the quality of the currently recorded information. Please refer to the technical notes for further details.

Ranked frequencies of injury hospitalisations and deaths by cause remained stable since the last financial year of reporting.



Falls remain the leading cause of injury for hospitalisations (2023–24) and deaths (2022–23)

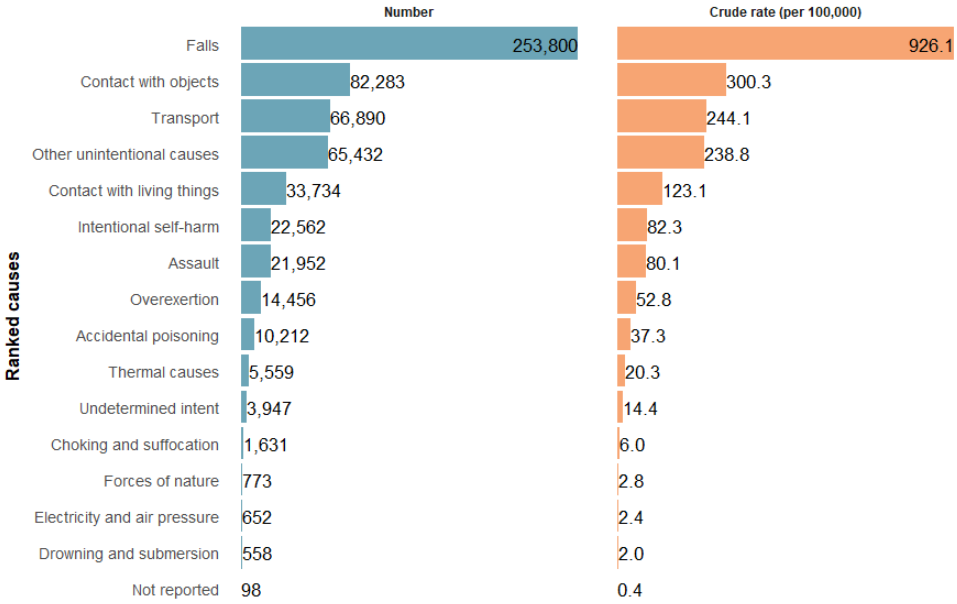
In 2024–25, the leading causes of injury hospitalisations were (Figure 4):

1. falls (253,800 hospitalisations or 926.1 per 100,000 population)
2. contact with objects (82,283 cases or 300.3 per 100,000 population)
3. transport (66,890 cases or 244.1 per 100,000 population)

In 2023–24, the leading causes of injury deaths were (Figure 5):

1. falls (6,722 deaths or 25.0 per 100,000 population)
2. suicide (3,316 deaths or 12.3 per 100,000 population)
3. accidental poisoning (1,597 deaths or 5.9 per 100,000)

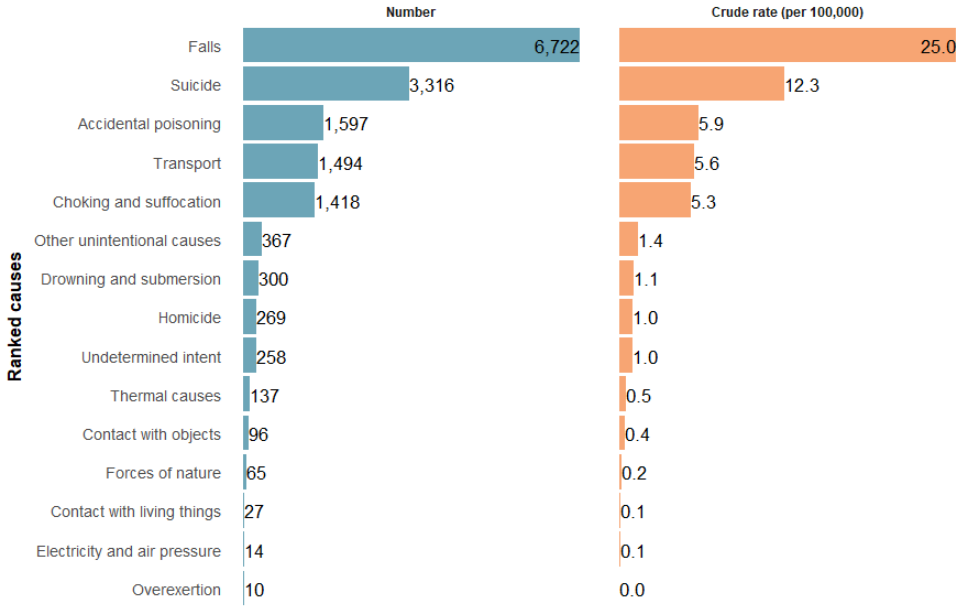
Figure 4: Injury hospitalisations by cause of injury, 2024–25



Note: Numbers and crude rates of hospitalisations (per 100,000 population) are represented by ranked rows.

Sources: AIHW National Hospital Morbidity Database and ABS National, state and territory population.

Figure 5: Injury deaths by cause of injury, 2023–24



Note: Numbers and crude rates of deaths (per 100,000 population**)** are represented by ranked rows.

Sources: AIHW National Mortality Database and ABS National, state and territory population.

For more detail, see Supplementary data tables H2 and D2.

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Severity

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Emergency department presentations

On this page:

- How do people get to and leave Emergency Departments?
- When do injuries present to Emergency Departments?
- Urgency of care
- Waiting times

How do people get to and leave Emergency Departments?

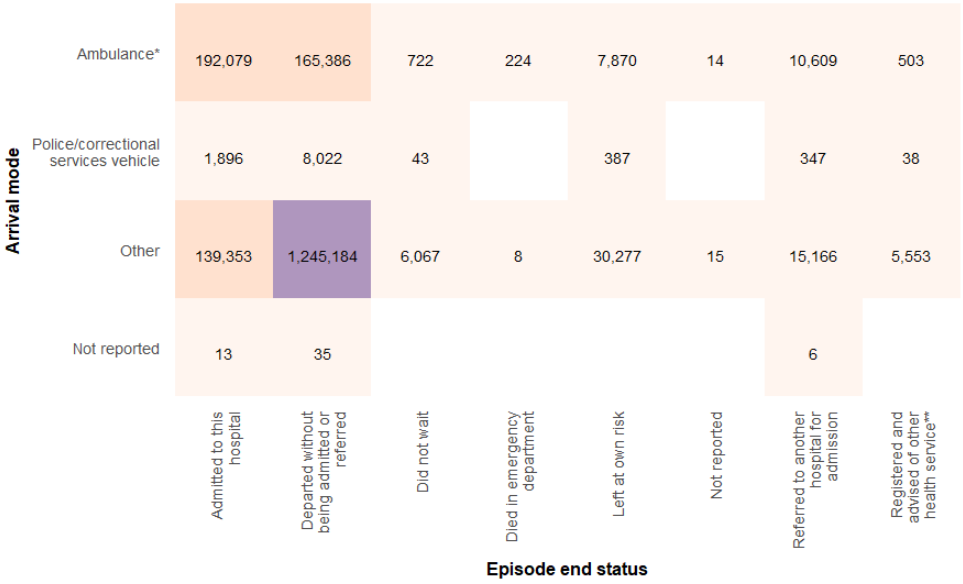
How a person arrives at and departs from an emergency department (ED) can provide information about the severity of their presentation. Presentations to ED are categorised into three arrival modes:

- Ambulance, air ambulance or helicopter rescue service
- Police/correctional services vehicle
- Other (includes people who either walked into ED, or came by private transport, public transport, or taxi)

In 2024–25, Other was the most common form of arrival to ED with an injury (78.8% or 1,441,624 cases), followed by Ambulance, air ambulance or helicopter rescue service at 377,407 cases (20.6%) (Figure 6). Comparing to the corresponding proportions for all ED presentations (72.7% for *Other* and 26.6% for *Ambulance* in 2024–25), injury presentations were less likely to arrive to an ED by ambulance services.

In 2024–25, 77.5% of ED presentations for injuries departed without being admitted or referred to another hospital. A moderate proportion of presentations for injuries did require further hospital treatment, with 18.2% being admitted to the same hospital they presented to.

Figure 6: ED presentations for injury by arrival mode and episode end status, 2024–25



Notes:

1. Cells contain numbers of injury related ED presentations, by arrival mode and episode end status.

* 'Ambulance' includes air ambulance and helicopter services

** In these cases, the person was registered, advised of another health care service, and left without being attended to.

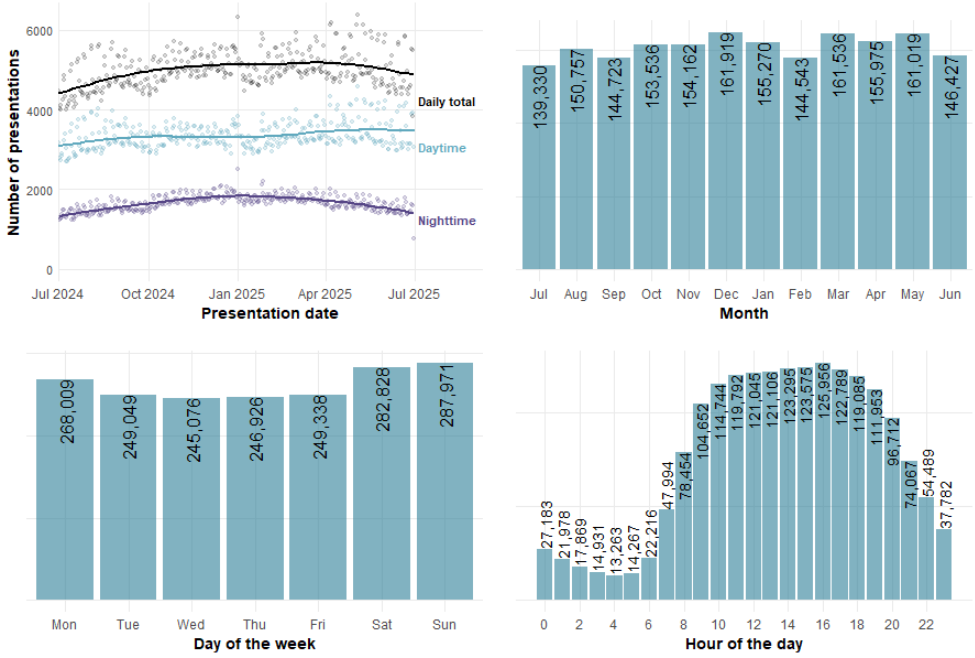
Source: AIHW National Non-admitted Patient Emergency Department Care Database.

When do injuries present to Emergency Departments?

In 2024–25 (Figure 7):

- injury presentations to ED peaked in December at 161,918 cases
- presentations at nighttime (18:00 to 06:00) were more common in summer months, and peaked on January 1st
- weekends were generally busier than weekdays. Sunday (15.8% of presentations) followed by Saturday (15.5% of presentations) were the busiest days of the week for injury presentations to ED
- patients tended to present to ED for injury during the day (business hours) with a peak in the afternoon around 4pm. Most presentations (77%) occurred between 8am and 8pm daily. On average, 4am was the quietest time of day.

Figure 7: ED presentations for injury, by date, month, day of the week and hour of the day, 2024–25



Note: Top left panel: Daytime is 06:00 to 17:59, nighttime is 18:00 to 05:59.

Source: AIHW National Non-admitted Patient Emergency Department Care Database.

Urgency of care

Every presentation to an ED is assigned one of 5 triage categories based on the urgency with which the person requires assistance (Table 1).

Table 1: Description and response for triage categories in emergency departments

Category	Description	Response
1- Resuscitation	Immediately life threatening	Immediate simultaneous assessment and treatment
2 - Emergency	Imminently life threatening OR Important time critical treatment OR Very severe pain	Assessment and treatment within 10 minutes
3 - Urgent	Potentially life threatening OR Situational urgency OR Humane practice mandates the relief of severe discomfort or distress within 30 minutes	Assessment and treatment start within 30 minutes
4 - Semi-urgent	Potentially serious OR Situational urgency OR Significant complexity or severity OR Humane practice mandates the relief of discomfort or distress within one hour	Assessment and treatment start within 60 minutes

5 - Non-urgent	Less urgent OR clinic-administrative problems	Assessment and treatment start within 120 minutes

The bulk of injury ED presentations were triaged as semi-urgent (Table 2). In 2024–25, out of the 1.8 million ED presentations for injury, almost half (48.6%) were triaged as semi-urgent and a further 9.2% as non-urgent. This was higher than the proportion of ED presentations for all conditions that were semi-urgent (33.6%) or non-urgent (6.1%), indicating that injuries that presented to ED were generally less severe than presentations overall.

Table 2: ED presentations for injury by triage category, 2024–25

Triage category	Number of injury presentations	Percentage (%)	Corresponding percentage of all ED presentations (%)*
Resuscitation	18,690	1.0	1.0
Emergency	206,460	11.3	17.7
Urgent	547,719	29.9	41.7
Semi-urgent	888,728	48.6	33.6
Non-urgent	168,194	9.2	6.1
Total	1,829,829	100.0	100.0

Notes:

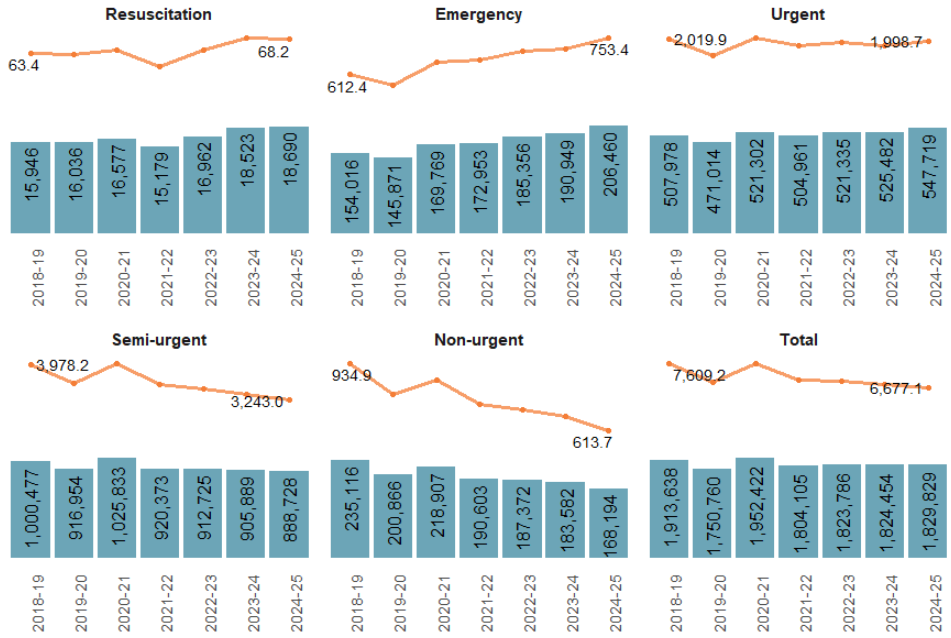
* calculated from Supplementary data table 4.2, Emergency department presentations, by triage category and arrival mode, states and territories, 2024–25, MyHospitals Emergency department Care 2024–25 data tables.

1. Due to rounding, column percentage totals may not sum to 100%.

Sources: AIHW National Non-admitted Patient Emergency Department Care Database.

Over the last 7 years from 2018–19, the rates of ED presentations by triage category have fluctuated, with presentations triaged as Emergency rising and those triaged as Semi-urgent and Non-urgent falling over time (Figure 8).

Figure 8: ED presentations for injury by triage category, 2018–19 to 2024–25

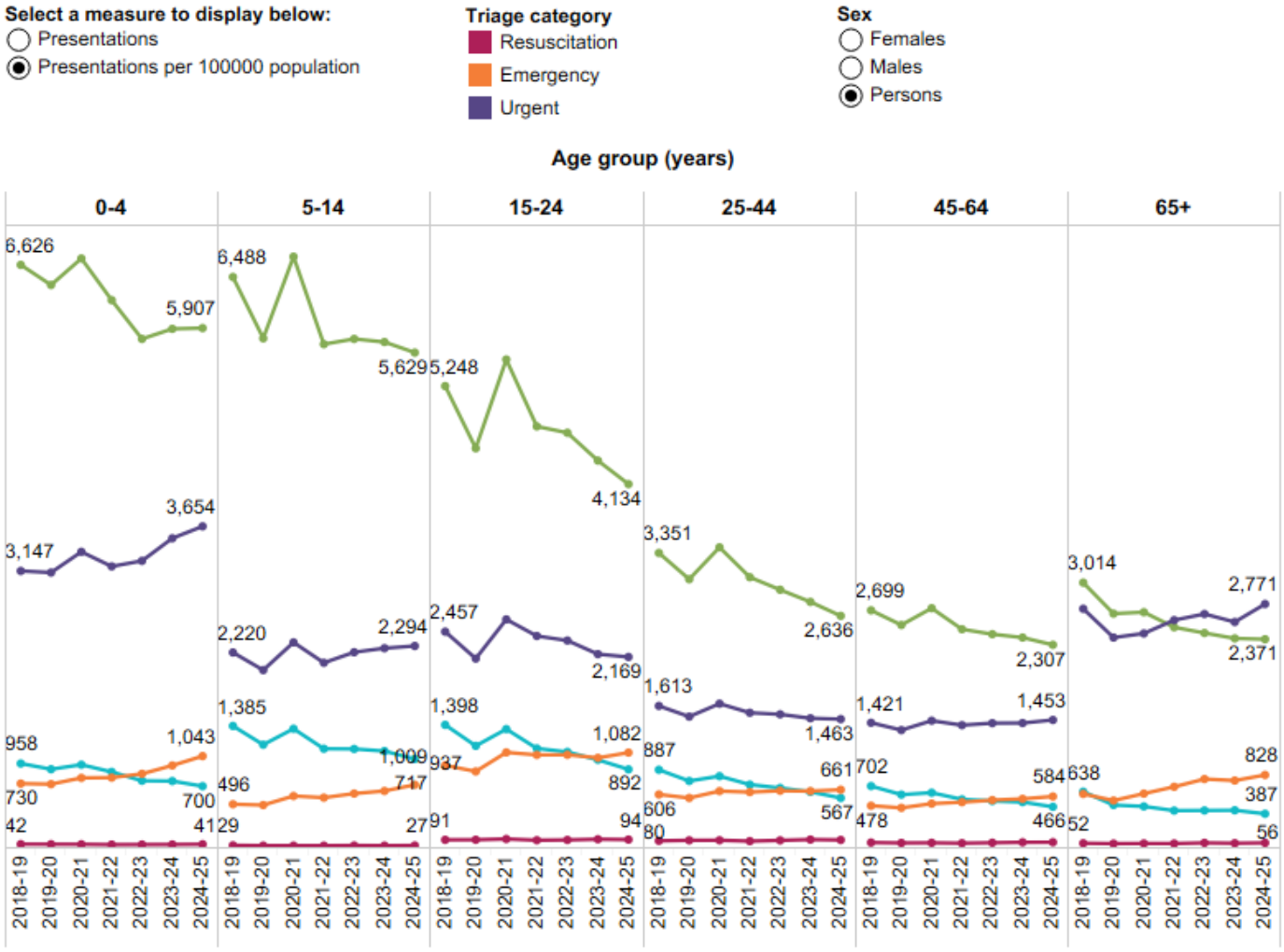


Note: Columns are case counts, line graphs present age-standardised rate per 100,000 population.

Sources: AIHW National Non-admitted Patient Emergency Department Care Database, state and territory population.

Semi-urgent presentations are the most common across all age groups but have decreased over time, with working age adults having the lowest rates of Semi-urgent injury presentations to ED (Figure 9). In contrast, Emergency injury presentation rates have increased for all age groups, with the largest increases recorded in people aged 14 and under.

Figure 9: ED presentations for injury by triage category, sex and age group, 2018–19 to 2024–25

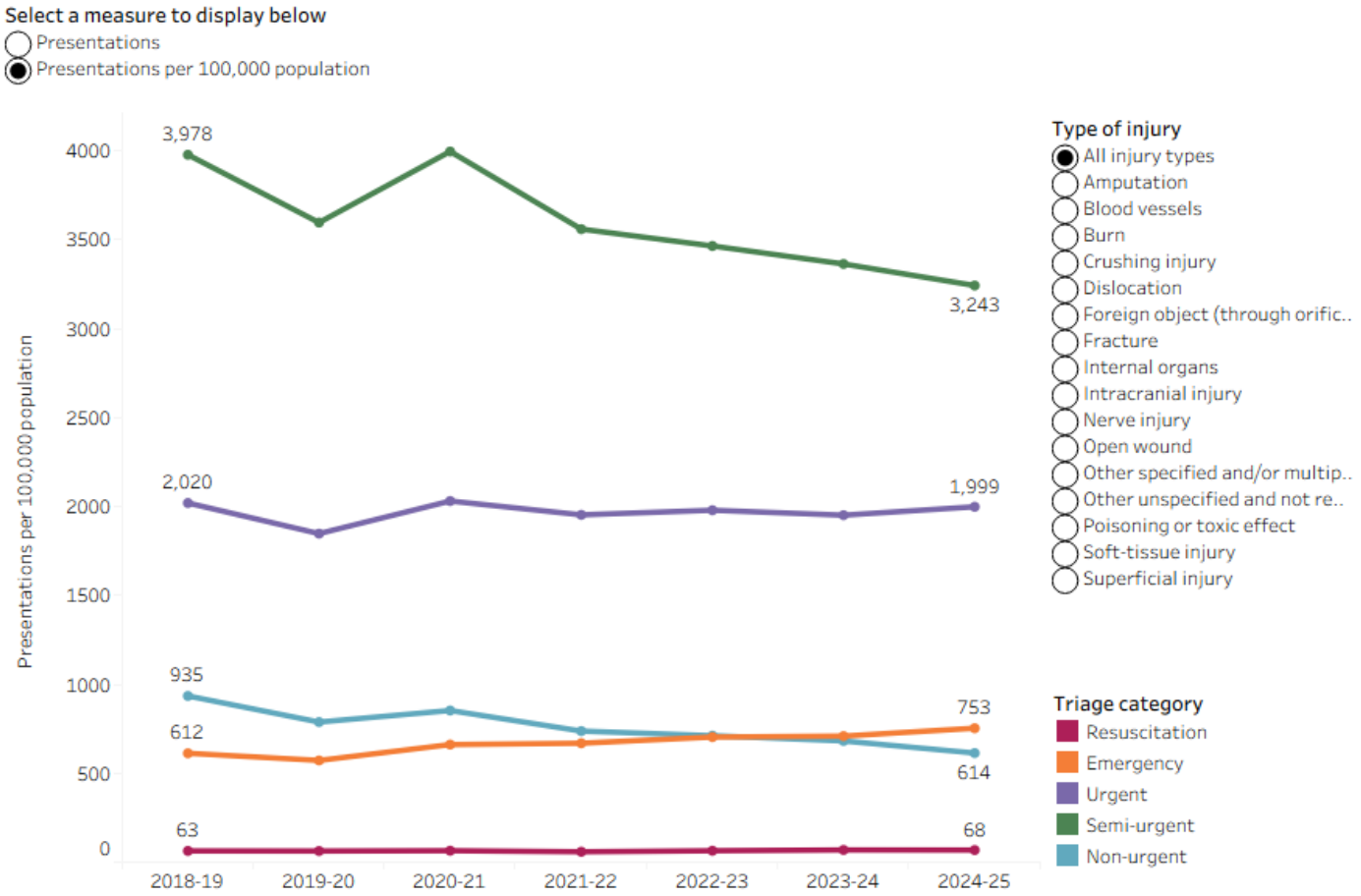


Note: Crude rates per 100,000 population.

Sources: AIHW National Non-admitted Patient Emergency Department Care Database, state and territory population.

The type of injury determines the required care and corresponding triage category (Figure 10). Most injuries are triaged as Semi-urgent, while dislocations, internal organ and intracranial injuries were usually Urgent. In 2024-25, amputations and poisoning (or toxic effect) ED presentations were typically triaged at the Emergency level.

Figure 10: ED presentations for injury by triage category and type of injury, 2018–19 to 2024–25



Note: Crude rates per 100,000 population.

Sources: AIHW National Non-admitted Patient Emergency Department Care Database, state and territory population.

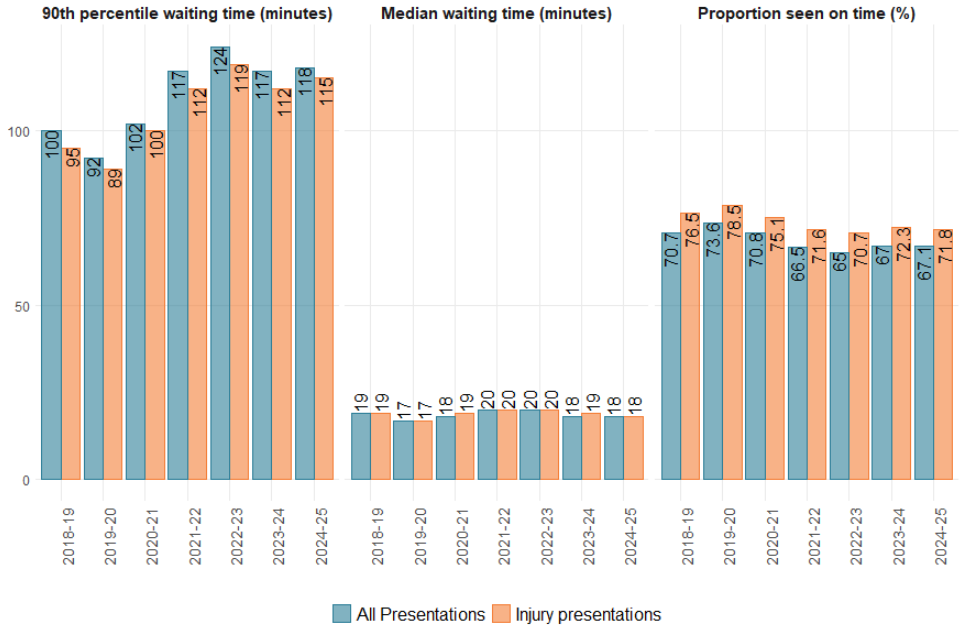
Waiting times

In 2024–25:

- 50% of patients who presented to ED for injury were seen within 18 minutes. This waiting time was equal to the average waiting time for patients who presented to ED for all types of conditions (18 minutes)
- 90% of injury patients were seen within 1 hour and 55 minutes, a similar waiting time to 1 hour and 58 minutes for patients who presented to ED for any type of condition
- 71.8% of patients treated in ED for injury were 'seen on time', compared with 67.1% of patients treated in ED for all conditions.

Over the last 7 years, the overall proportion of injury ED presentations seen on time has decreased, as has the comparable proportion for all conditions. In each year over the same period, those presenting to ED for injuries were slightly more likely to be seen on time than those for all conditions (Figure 11).

Figure 11: Median and 90th percentile waiting time (minutes) of ED presentations and proportion seen on time, 2018–19 to 2024–25



Notes:

1. Median waiting time is equivalent to the time within which 50% of patients were seen; 90th percentile waiting time is equivalent to the time within which 90% of patients were seen.
2. Data relate to presentations with a type of visit of Emergency presentation only.

Sources: AIHW National Non-admitted Patient Emergency Department Care Database, state and territory population.

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Hospitalisations

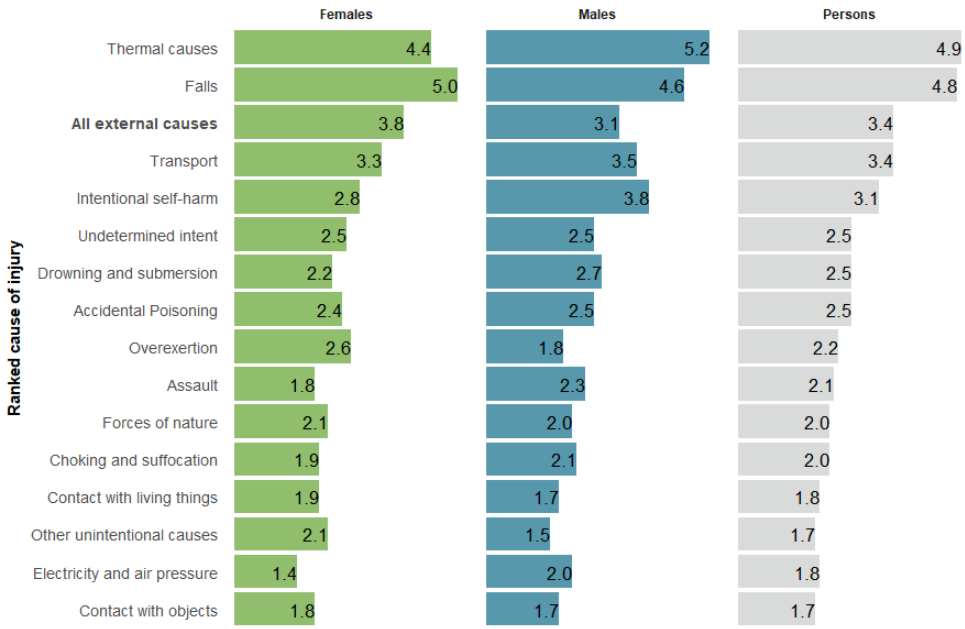
There are many ways that the severity, or seriousness, of an injury can be measured. Some of the ways to measure the severity of hospitalised injuries are:

- number of days in hospital (length of stay)
- time in an intensive care unit (ICU)
- time on a ventilator
- in-hospital deaths.

Average length of hospital stay

The most severe injuries in 2024–25 as measured by average length of stay in hospital were due to thermal causes (4.9 days), falls (4.8 days) and transport (3.4 days) (Figure 12).

Figure 12: Average length of stay (days) by injury cause and sex, 2024–25

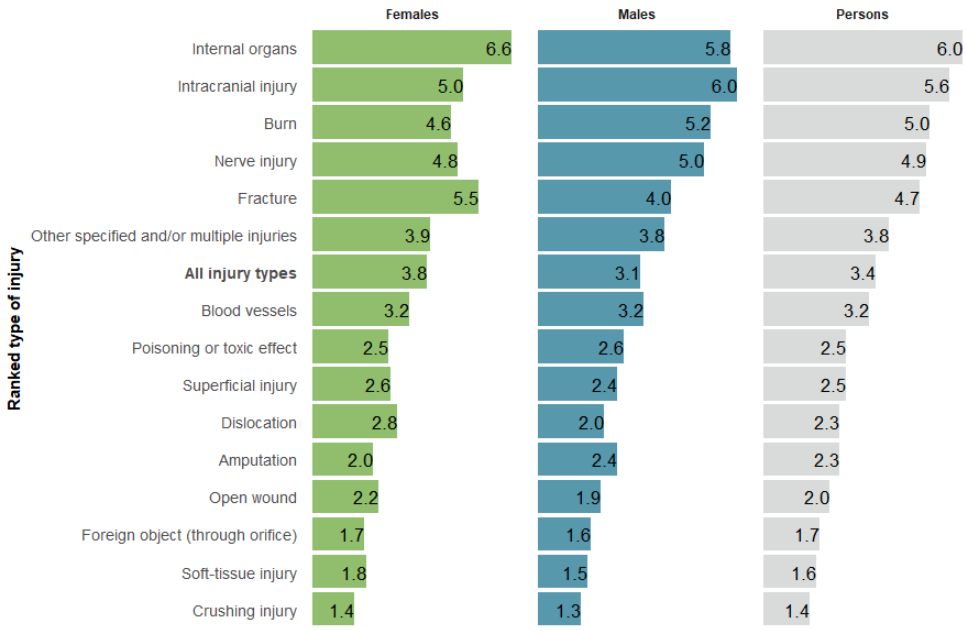


Source: AIHW National Hospital Morbidity Database

For the most severe injury types, average hospital stays were 6.0 days for internal organ injuries, 5.6 days for intracranial injuries, and 5.0 days for burns.

The longest average length of stay for males was for intracranial injuries (6.0 days), and for females was for internal organ injuries (6.6 days) (Figure 13).

Figure 13: Average length of hospital stay (days) by injury type and sex, 2024–25



Source: AIHW National Hospital Morbidity Database.

ICU, CVS and in-hospital deaths

An intensive care unit (ICU) is a designated ward of a hospital which is specially staffed and equipped to provide observation, care and treatment to patients with actual or potential life-threatening illnesses, injuries or complications, from which recovery is possible (METEOR 2018).

In the ICU, patients may need continuous ventilatory support (CVS), or 'life support', when a patient breathes via an artificial airway with the aid of a machine.

In 2024–25:

ICU
12,216 injury cases were admitted to intensive care units (about 2.1%, an ASR of 42.3 per 100,000 people)

CVS
7,012 injury cases received continuous ventilatory support (about 1.2%, an ASR of 25.6 per 100,000 people)

In-hospital deaths
3,411 hospitalised injury cases died in hospital (about 0.6%, an ASR of 9.4 per 100,000 people)

More males were admitted to ICU than females (54.8 and 32.3 per 100,000 population, respectively) (Table 3).

Although more females are hospitalised for falls (Figure 18) males have a higher ICU admission rate for falls (15.9 and 10.4 per 100,000, respectively), suggesting that fall-related injuries may be more severe in males.

For transport-related injuries, males were 3.1 times more likely to be admitted to ICU compared with women (13.8 and 4.5 per 100,000, respectively). More females were admitted to the ICU for intentional self-harm compared with males (10.6 and 9.3 per 100,000, respectively).

Table 3: Injury hospitalisations with ICU admission, by external cause of injury, 2024–25

External cause of injury	Males	Females	Persons
All external causes	**7,456**	**4,450**	**11,924**
Falls	2,164	1,440	3,606
Intentional self-harm	1,266	1,455	2,726
Transport	1,873	626	2,506
Accidental poisoning	638	363	1,003
Undetermined intent	315	166	481
Assault	381	79	462
Contact with objects	269	75	344
Other unintentional causes	164	103	267
Thermal causes	132	41	173
Contact with living things	116	51	167
Choking and suffocation	47	21	68
Drowning and submersion	45	12	57
Overexertion	21	16	37

Note: Rows with under 20 persons are not displayed

Source: AIHW National Hospital Morbidity Database

Males were around twice as likely to require CVS compared with women (32.5 and 16.5 per 100,000 population, respectively) (Table 4). Injuries due to intentional self-harm (7.7 per 100,000), transport (5.3 per 100,000) and falls (4.1 per 100,000) were most likely to result in continuous ventilatory support.

Table 4: Injury hospitalisations with receipt of CVS, by external cause of injury, 2024–25

External cause of injury	Males	Females	Persons
All external causes	**4,425**	**2,271**	**6,698**
Intentional self-harm	1,060	1,040	2,101
Transport	1,117	345	1,463
Falls	807	319	1,126
Accidental poisoning	499	264	763

Undetermined intent	276	143	419
Assault	275	42	317
Contact with objects	153	29	182
Thermal causes	86	28	114
Other unintentional causes	45	18	63
Choking and suffocation	38	17	55
Drowning and submersion	30	10	40
Contact with living things	18	11	29

Note: Rows with under 20 persons are not displayed

Source: AIHW National Hospital Morbidity Database

Injuries caused by falls were the leading cause of deaths in hospital (9.6 per 100,000 population), followed by transport and intentional self-harm. Males were more likely to die in the hospital than females (14.1 and 10.1 per 100,000, respectively) (Table 5).

Table 5: Injury deaths during hospitalisations, by external cause of injury, 2024–25

External cause of injury	Males	Females	Persons
All external causes	**1,912**	**1,396**	**3,309**
Falls	1,435	1,185	2,620
Transport	175	61	237
Intentional self-harm	100	54	154
Other unintentional causes	41	24	65
Assault	37	7	44
Choking and suffocation	23	18	41
Contact with objects	24	13	37
Undetermined intent	26	6	32
Accidental poisoning	18	9	27
Thermal causes	16	7	23

Note: Rows with under 20 persons are not displayed

Source: AIHW National Hospital Morbidity Database

References

METEOR (2018), Intensive Care Unit – Metadata 327234, AIHW, accessed 18 May 2026.

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Types of injury

Injuries can be classified into several common injury types, including:

- **Fracture**: A partial or complete break in a bone.
- **Soft-tissue injury**: Sprain or strain of muscles, ligaments or joints.
- **Open wound**: A break in the skin such as a cut, puncture or bite.
- **Superficial injury**: An injury to the skin surface such as abrasion, bruising or blistering.
- **Poisoning or toxic effect**: Exposure to a substance that causes harmful physiological effects.

See Supplementary data tables E3 and H9 for all injury types.

Fractures were the most common type of injury seen in ED presentations (406,819 cases, 1,484.5 per 100,000 population). This was followed by open wounds (302,184 cases, 1,102.7 per 100,000) and soft-tissue injuries (206,589 cases or 753.9 per 100,000).

Different patterns were evident by age group (Figure 14). Where the injury type was recorded:



Children aged 0–4

Open wounds were the most commonly recorded injuries for those aged 0–4, followed by superficial injuries and fractures



Children aged 5–14

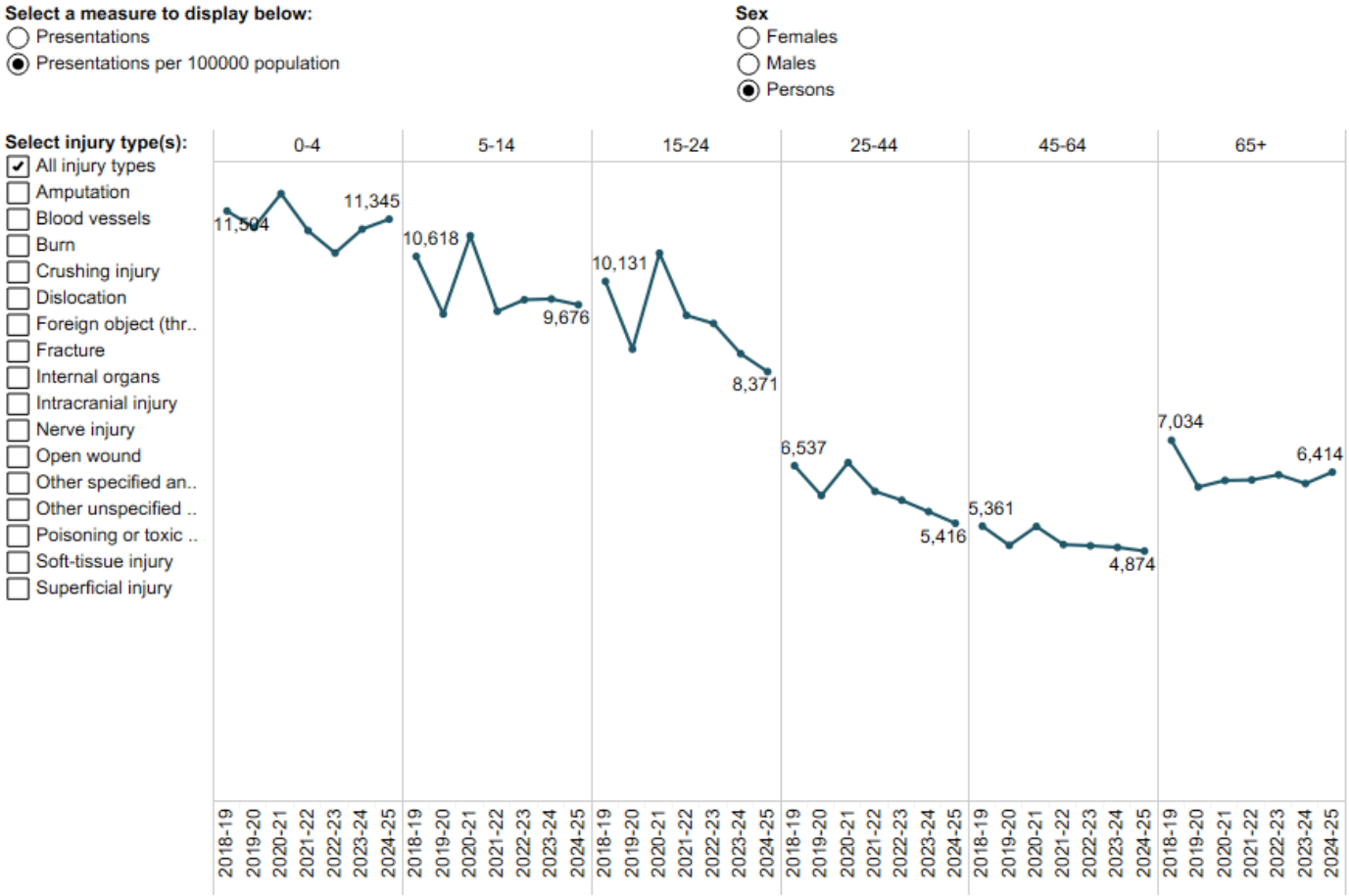
Those aged 5–14 were most likely to present to ED for fractures, followed by soft-tissue injuries and open wounds



Adults aged 65 and over

In the older age groups of 65 and above, fractures were the most common injury type, followed by open wounds and superficial injuries

Figure 14: Injury ED presentations, by age group, sex and type of injury, 2018–19 to 2024–25



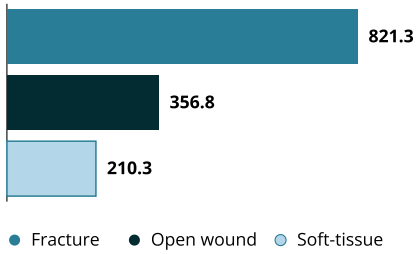
Source: AIHW National Hospital Morbidity Database.

Hospitalisations 2023-24

Across all ages, the top 3 types of injuries were:

- fractures
- open wounds
- soft-tissue injuries

(Chart showing age-standardised rates per 100,000 population)



Injury hospitalisations were most likely to be for fractures in 65+ year-olds, at a rate of 1,968.9 per 100,000 (93,887 cases) .

For more information please see Supplementary data tables E3 and H9.

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Body parts injured

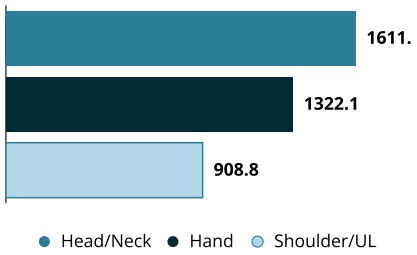
Emergency Department presentations

ED presentations 2023-24

The three most frequent body parts injured in ED presentations were

1. the head and neck
2. the wrist and hand
3. the shoulder and upper limb (UL)

Chart presents age-standardised rates per 100,000 population



The most common combination of injury types and body parts injured (where both were specified) were:

- fracture to the shoulder and upper limb (excluding wrist and hand) (a rate of 450.6 per 100,000 population)
- open wound to the wrist and hand (395.7 per 100,000)
- open wound to the head and neck (369.1 per 100,000)

The most common principal diagnoses for injury ED presentations varied across age groups.

Children aged 0–4

The most common ED principal diagnosis in children aged 0–4 in 2023–24 was *open wound of head.*

Children aged 5–14

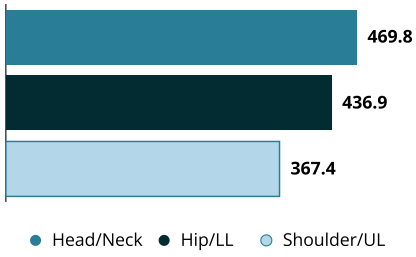
The most common ED principal diagnosis in children aged 5–14 in 2023–24 was *fracture at wrist and hand level*.

Adults aged 25–44

The most common ED principal diagnosis in adults aged 25–44 in 2023–24 was *injury of unspecified body region*.

For more information see Supplementary data tables E2 and E4.

Injury hospitalisations



Injury hospitalisations 2023-24

Across all ages, the top three body parts injured were:

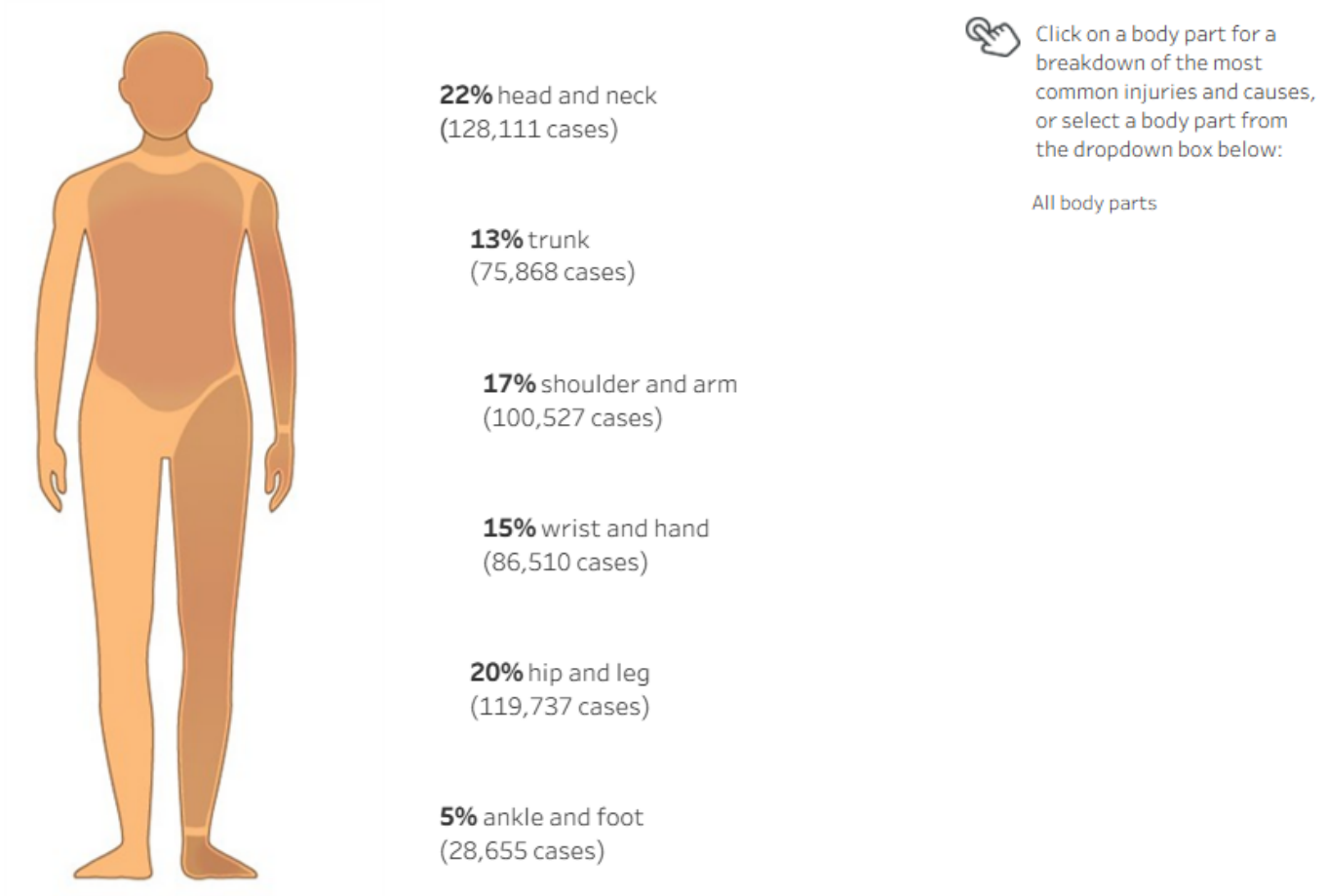
1. head and neck
2. hip and lower limb (LL)
3. shoulder and upper limb (UL)

Chart shows age-standardised rates per 100,000 population. See Figure 16 for more detail.

Injury hospitalisations among people aged 65 and over were most likely to be for:

1. head and neck injuries (crude rate of 1,145.0 per 100,000 population)
2. hip and lower limb injuries (crude rates of 1,115.5 per 100,000 population)

Figure 15: Injury hospitalisations by main body part injured, 2024–25



Note: Cases of hospitalisations are represented at person level with highlighted body parts.

Source: AIHW National Hospital Morbidity Database

Persons aged 65 and over and young children aged 0-4 were the most likely to sustain injuries to the head and neck (1,107.7 and 784.5 per 100,000, respectively).

Overall, persons aged 65 and over reported the highest crude rates of injuries across all body parts apart from the ankle and foot, injuries not described in terms of body location, and the wrist and hand.

Young people aged 15-24 had the highest hospitalisation rates for:

- injuries to the ankle and foot – 125.9 per 100,000
- injuries not described in terms of body location – 229.5 per 100,000
- injuries to the wrist and hand – 449.6 per 100,000.

For hospitalisations in 2024–25, the most common combination of injury types and body parts injured (where both were specified) were:

- fracture to the shoulder and upper limb (excluding wrist and hand) (238.3 per 100,000)
- fracture to the hip and lower limb (excluding ankle and foot) (218.1 per 100,000)
- fracture to the trunk (thorax, abdomen, lower back, lumbar spine & pelvis) (157.1 per 100,000)

For more information see Supplementary data tables H10 and H11.

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Activity and place of occurrence

Place of occurrence

In 2024–25, 226,816 injury hospitalisation records (about 38.8%) had place missing.

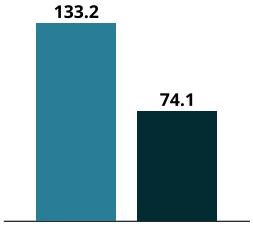
Information around place of occurrence of injuries is only reported for hospitalisations, and large proportions of records have missing information.

For the 61.2% of hospitalised injury cases where this information was available, the home was the most common place of injury (175,398 cases) followed by streets or highways (58,659 cases), sports areas (30,826 cases) and aged care facilities (27,012 cases).



In the home, 2023–24

Injuries in the home were most likely among those aged 65 and over, at a crude rate of 2,033.2 injury hospitalisations per 100,000 population (94,140 cases).



In aged care facilities, 2023–24

Females were almost twice as likely to be injured in an aged care facility and hospitalised than males.

Chart shows age-standardised rates per 100,000 population



Streets and highways, 2023–24

The highest rates of injuries in streets and highways were among 65+ year-olds (300.9 per 100,000 population) and 15–24 year-olds (277.8 per 100,000 population).

Activity at the time of injury

In 2024–25, 393,164 injury hospitalisation records (about 67.3%) had activity information missing.

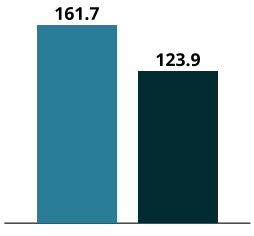
Information around activity undertaken at the time of injury is only reported for hospitalisations, and large proportions of records have missing information.

For the 32.7% of hospitalised injury cases where this information was available, sport was the most common activity (60,974 cases) followed by daily living activities (38,732 cases), unpaid work and learning (35,663 cases) and leisure (22,798 cases).



Sport

Sport-related injuries in 2023–24 were most likely among 15–24-year-olds (506.9 per 100,000 population)



Daily living activities

Females had higher rates of injury hospitalisation while undertaking daily living activities compared to males.

Chart shows age-standardised rates per 100,000 population.

Daily living activities include resting, sleeping, eating and personal hygiene.

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Differences by sex and age groups

Emergency Department (ED) presentations

Injury hospitalisations

Injury deaths

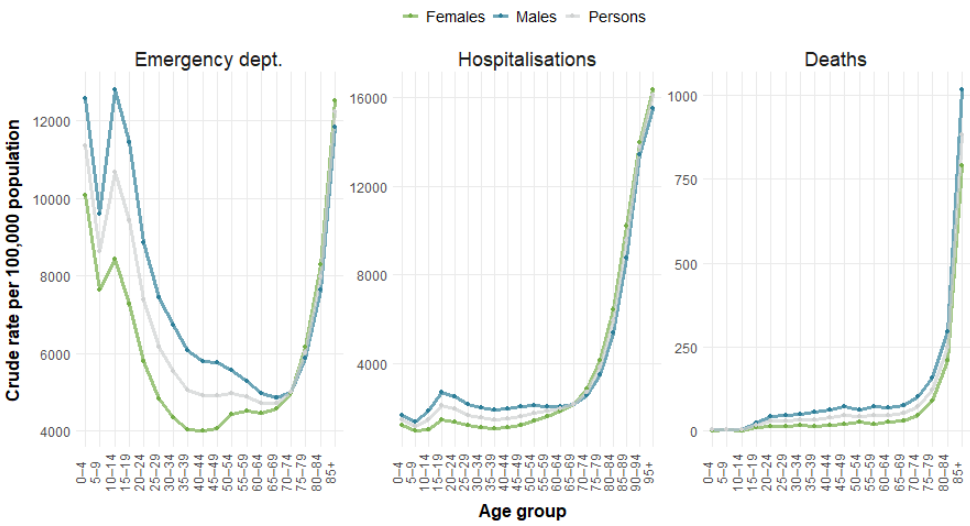
Overview

Different groups of the population experience differing rates of injury by cause and type of injury. Age can affect injury risk factors, in part due to the life-stages a person goes through, including their physical development, health status, socioeconomic area, lifestyle, and activities a person is involved in (Chau et al 2009). While males contribute to most cases of injury across most causes, older people (particularly females) and young children are more prone to falls. This chapter details how causes of injury differ by age and sex.

Overall, males had higher rates of injury-related ED presentations, hospitalisations, and deaths than females (Figure 16). For example:

- Young males were more likely than females to present to ED and be hospitalised, however this difference narrowed among those aged 65 and above.
- From about age 10 onward, males consistently had higher injury-related death rates across all age groups. Female injury rates approached those of males later in life – around age 70 for ED visits, 65 for hospitalisations, and 80 for deaths.
- Injury rates were high among older adults and children, with ED presentations notably elevated among children under 14 before declining during adolescence.

Figure 16: ED presentations, hospitalisations (2024–25), and deaths (2023–24) for injury, by 5-year age group

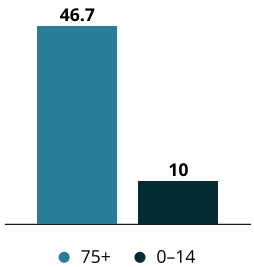


Notes:

1. Person totals include hospitalisations where the sex of the patient was other, inadequately described or not stated.
2. Crude rates per 100,000 population.

Sources: AIHW National Hospital Morbidity Database, AIHW National Mortality Database, AIHW National Non-admitted Patient Emergency Department Care Database, and ABS National, state and territory population.

Key findings - Causes of injury hospitalisations in 2024–25



Falls

Falls injury hospitalisations were most common among those aged 75 and over, followed by those aged under 15 years.

Columns show percentage of all reported falls for each age group.



Transport

3 in 10 transport injury hospitalisations in 2023–24 occurred among 15-29 year-olds (28.3%)



Accidental poisoning

Accidental poisoning in 2023–24 mainly affected the very young (0-to-4-year-olds) and the elderly (80 years and older)



Assault

Most assault-related injury hospitalisations in 2023–24 occurred among 15-to-49-year olds (77.1%)

Key findings - Causes of injury deaths in 2023–24



Transport

Transport-related injury deaths in 2022–23 were highest among 20-24-year-olds (134 deaths)



Choking and suffocation

Choking and suffocation was the second highest cause of injury death in 2022–23 for persons aged 85 and over (540 deaths)



Suicide was the leading cause of injury death for those aged between 15 and 64 years.

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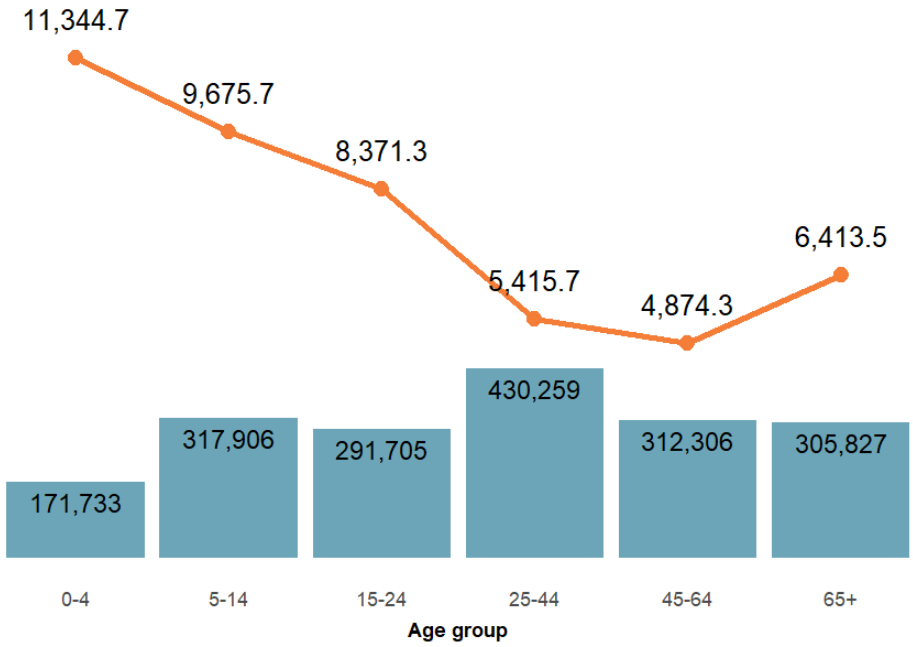
Emergency department presentations

As the external cause of injury cannot be determined for ED presentations (see Technical notes for details), we can only provide information on the impact of age and sex on ED presentations for injury as a whole, and are unable to distinguish between specific causes of injury.

More males presented to ED with injuries than females (see Supplementary data Table E1), however the age patterns for ED presentations differed to those for hospitalisations or deaths (Figure 17).

Younger people had higher ED injury presentation rates than adults, with 0–4-year-olds at 11,344.7 per 100,000 persons, dropping to 4,874.3 per 100,000 persons for 45–64 year-olds, then rising to 6,413.5 for people aged 65 and over. In 2024–25, there were 781,344 presentations for people under 24 (42.7% of the total 1.8 million ED presentations), compared to 430,259 presentations for ages 25-44 (23.5%).

Figure 17: ED presentations for injury by age group, 2024–25



Note: Columns are case counts, while the line graph represents crude rates per 100,000 population.

Sources: AIHW National Non-admitted Patient Emergency Department Care Database and ABS National, state and territory population.

For more detail, see Supplementary data Table E3.

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Hospitalisations

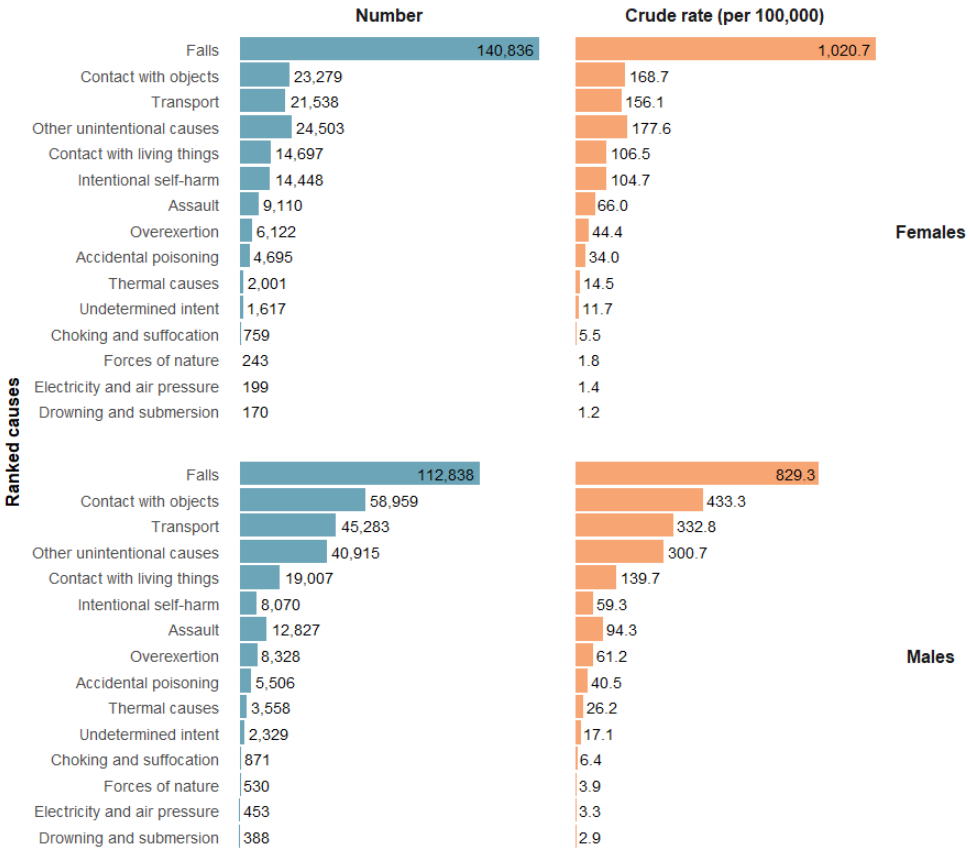
In 2024–25, 54.7% of injury hospitalisations were among males (319,928 cases, 2,351.3 per 100,000 population) and 45.2% among females (264,249 cases, 1,915.2 per 100,000).

Males were more likely to be injured and hospitalised across all causes except for:

- falls (1,020.7 females and 829.3 males per 100,000)
- intentional self-harm (104.7 females and 59.3 males per 100,000) (Figure 18)

Males were hospitalised nearly 2.6 times more frequently than females due to injuries from contact with objects. Likewise, for transport-related injuries, males were hospitalised at more than twice the rate of females.

Figure 18: Injury hospitalisations by injury cause and sex, 2024–25



Note: Causes are ranked by descending number of hospitalisations for all persons (Figure 4)

Sources: AIHW National Hospital Morbidity Database and ABS National, state and territory population.

Hospitalisations and deaths due to injuries generally increased with age for both sexes, reaching their peak rates after age 75 (Figure 16). Hospitalisation rates were slightly higher for adolescents and young adults (ages 10-20) for both sexes compared with younger children and working-age adults.

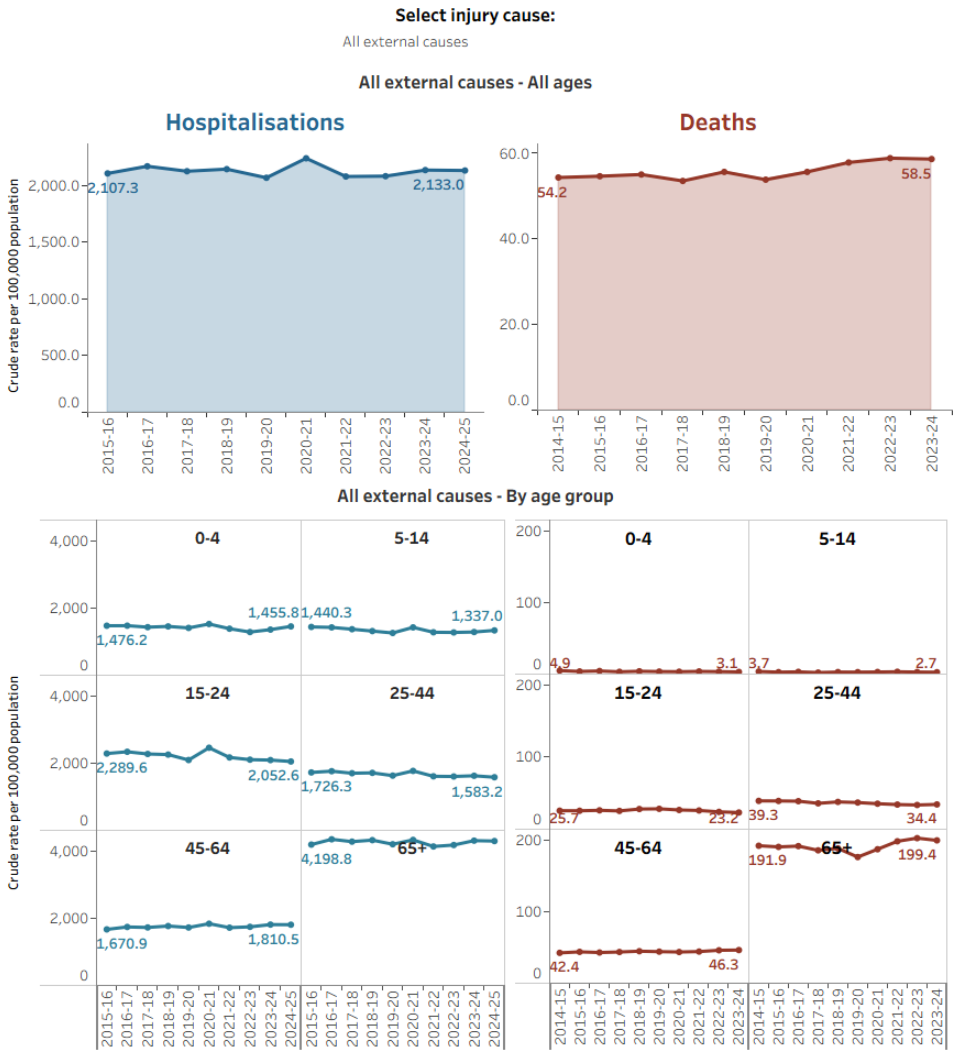
Males were more likely to be hospitalised with injuries than females across all age groups younger than 65, after which females were slightly more likely to be hospitalised. Across age groups, the highest numbers of cases overall were in males aged 15–19 (23,696 cases) and females aged 80–84 (21,749 cases).

For more detail, see Supplementary data Tables H1 and H2.

Trends by age

Over the past decade, rates of injury hospitalisation have remained steady or decreased slowly for most younger age groups but increased for adults aged 45 and over (Figure 19).

Figure 19: Trends in injury hospitalisations (2015–16 to 2024–25) and deaths (2014–15 to 2023–24) by age



The interactive chart shows injury hospitalisation and death trend data, by age group over the past decade.

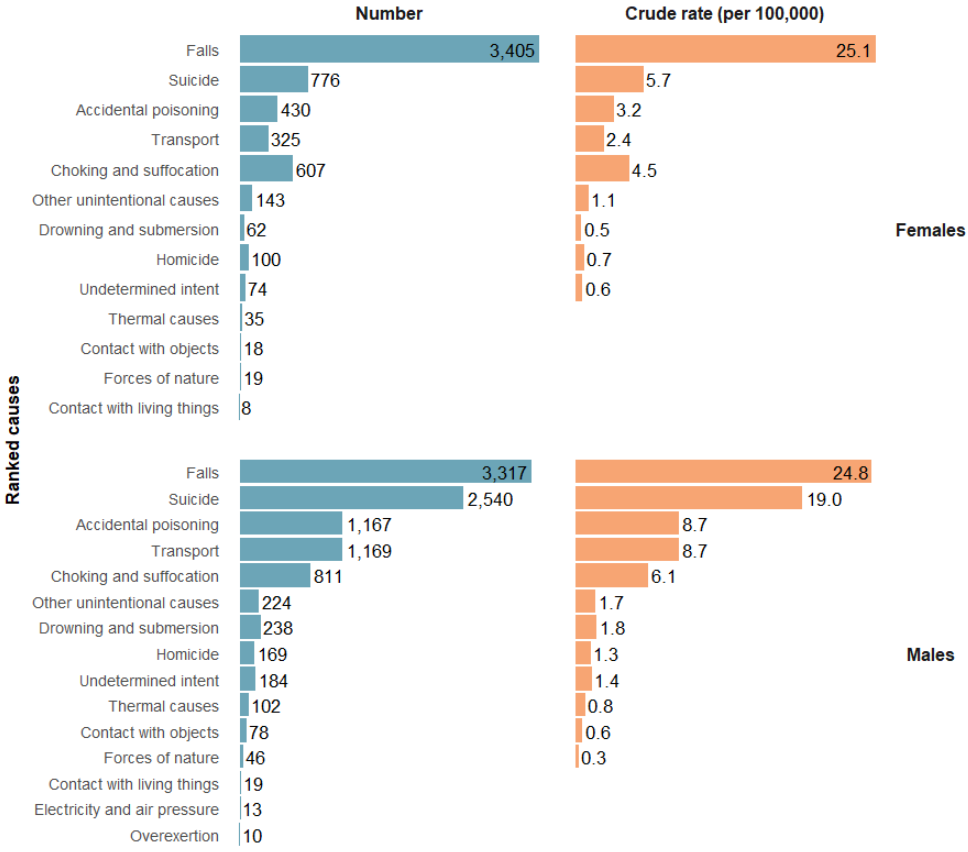
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Deaths

In 2023–24, around 3 in 5 injury deaths (62.4%) were for males (9,845 deaths, 73.6 per 100,000 persons) and 37.6% were for females (5,920 deaths, 43.7 per 100,000 persons).

Males were more likely to die from any cause of injury than females (Figure 20). Falls were the leading cause of death for both sexes.

Figure 20: Injury deaths by cause and sex, 2023–24



Note: Causes are ranked by descending total number of injury deaths (Figure 5).

Sources: AIHW National Mortality Database and ABS National, state and territory population.

The mortality rate increased with age for both sexes, and was higher among males than females from age 10 onward (Figure 16). Over half of injury deaths (54.2%) were for people aged 70 and over.

Trends by age

Over the past decade, injury mortality rates have remained steady or decreased for most younger age groups but increased for adults aged 45 and over (see Figure 19 on the previous page).

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First Nations people

Among Aboriginal and Torres Strait Islander (First Nations) people, there were (Figure 21):

149,811 ED presentations with a primary diagnosis of injury among First Nations people in 2023–24

- a crude rate of 14,538.3 presentations per 100,000
- 68,274 presentations for females (13,294.1 per 100,000)
- 81,502 presentations for males (15,761.6 per 100,000)

35,775 injury hospitalisations among First Nations people in 2023–24

- a crude rate of 3,471.8 per 100,000
- 16,317 for females (3,178.4 per 100,000)
- 19,443 for males (3,760.1 per 100,000)

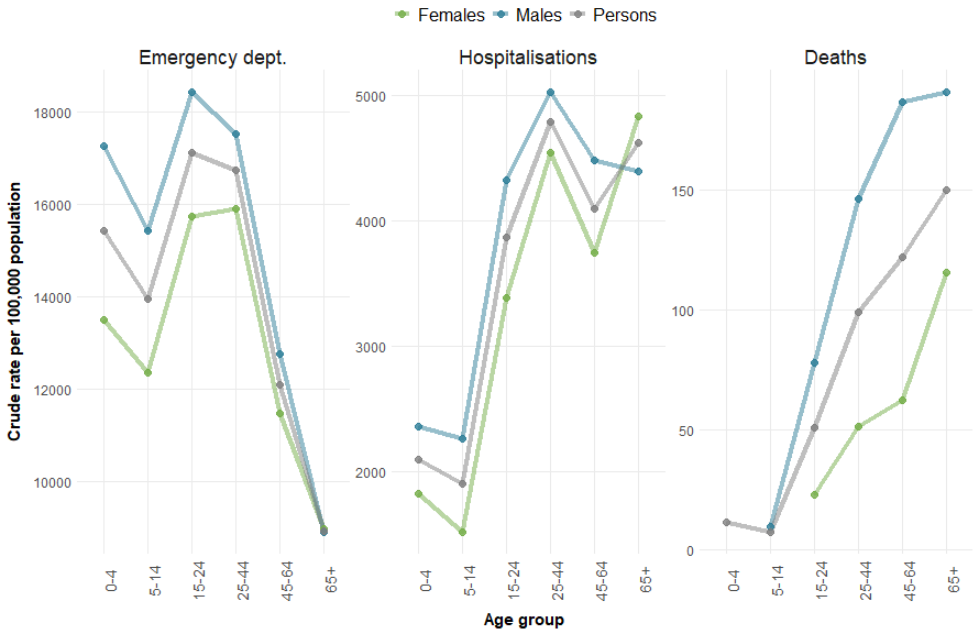
646 injury deaths among First Nations people in 2022–23

- a crude rate of 71.4 per 100,000
- 200 for females (44.2 per 100,000)
- 446 for males (98.5 per 100,000)

Deaths data only includes NSW, Qld, WA, SA and NT.

See Technical notes for details.

Figure 21: Injury ED presentations, hospitalisations (2024–25) and deaths (2023–24) among First Nations people by sex and age group



Notes:

1. Crude rates per 100,000 population.
2. Person totals include hospitalisations and ED presentations where the sex of the patient was other, inadequately described or not stated.
3. Deaths data only includes data for New South Wales, Queensland, Western Australia, South Australia, and the Northern Territory, as these jurisdictions are considered to have adequate levels of Indigenous identification in mortality data.

Sources: AIHW National Hospital Morbidity Database, AIHW National Mortality Database, AIHW National Non-admitted Patient Emergency Department Care Database, and ABS Estimates and Projections, Aboriginal and Torres Strait Islander Australians.

ED presentations

In 2024–25 there were 155,582 injury ED presentations among First Nations people – a rate of 14,823.4 per 100,000 population, or 18.5% of their total ED presentations.

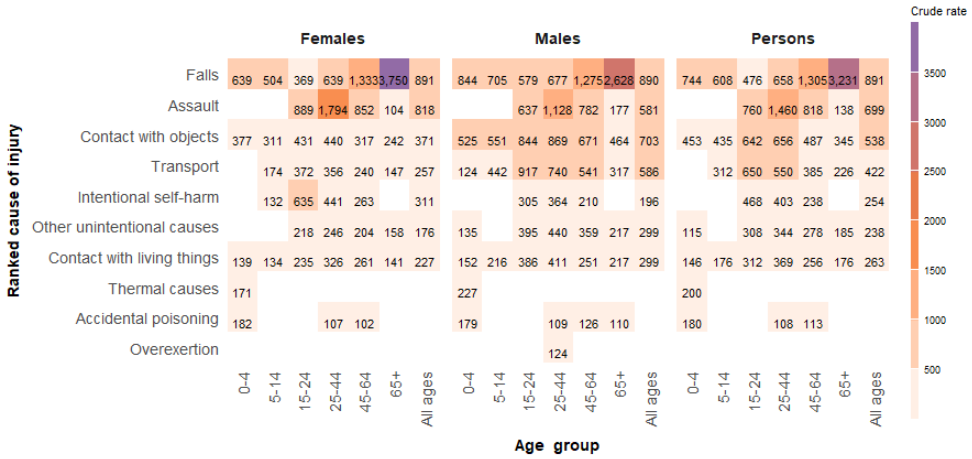
ED injury presentations varied by age: those aged 15–24 were most likely to present and people aged 65 and over were the least (Figure 21). As for all Australians, younger First Nations males presented to ED more frequently than females, but this difference was not observed among older adults aged 65 and over.

Most injury ED presentations were triaged as *Semi-urgent* (48.6%), 29.1% as *Urgent*, and 1.5% as *Resuscitation* (see Supplementary data Table E10). See Severity for general information on triage categories.

Hospitalisations

Falls, assault and contact with objects were the leading causes of injury-related hospitalisations among First Nations people, at rates of 890.5, 699 and 537.7 hospitalisations per 100,000 respectively. Falls were the top cause for both First Nations males and females (890.1 and 890.8 cases per 100,000, respectively) with the risk increasing with age (Figure 22).

Figure 22: Ranked causes of injury hospitalisation by age and sex among First Nations people, 2024–25



Notes:

1. Density map of crude rates of hospitalised cases of injury causes by age and sex.
2. Rates are crude rates per 100,000 population, rounded to a whole number.
3. Rates under 100 are not displayed.
4. Some external causes of injury are not displayed.

Sources: AIHW National Hospital Morbidity Database and ABS Estimates and Projections, Aboriginal and Torres Strait Islander Australians.

Comparing First Nations males and females:

In 2023–24 First Nations males were

- 1.2 times as likely to be hospitalised due to an injury, overall
- 2.2 times as likely to be hospitalised due to transport-related injuries

(compared to First Nations females)

In 2023–24 First Nations females were

- 1.7 times as likely to be hospitalised due to intentional self-harm
- 1.4 times as likely to be hospitalised due to assault

(compared to First Nations males)

The risk of injury varied by age among First Nations people (Figure 22). In general, those aged 25-44 had the highest rate of injury hospitalisations, with 4,788.2 cases per 100,000 population.

Deaths

The leading cause of death for First Nations people was suicide (23.3 per 100,000 population, Table 6).

Table 6: Top 6 ranked causes of death among First Nations people, by sex, 2023–24

	Number of deaths			Mortality rate		
External cause of injury	**Males**	**Females**	**Persons**	**Males**	**Females**	**Persons**
All external causes	504	197	701	97.5	38.4	68.0
Suicide	188	52	240	36.4	10.1	23.3
Accidental poisoning	99	49	148	19.2	9.5	14.4
Transport	90	32	122	17.4	6.2	11.8
Falls	38	35	73	7.4	6.8	7.1
Choking and suffocation	31	11	42	6.0	2.1	4.1
Homicide	24	12	36	4.6	2.3	3.5

Notes:

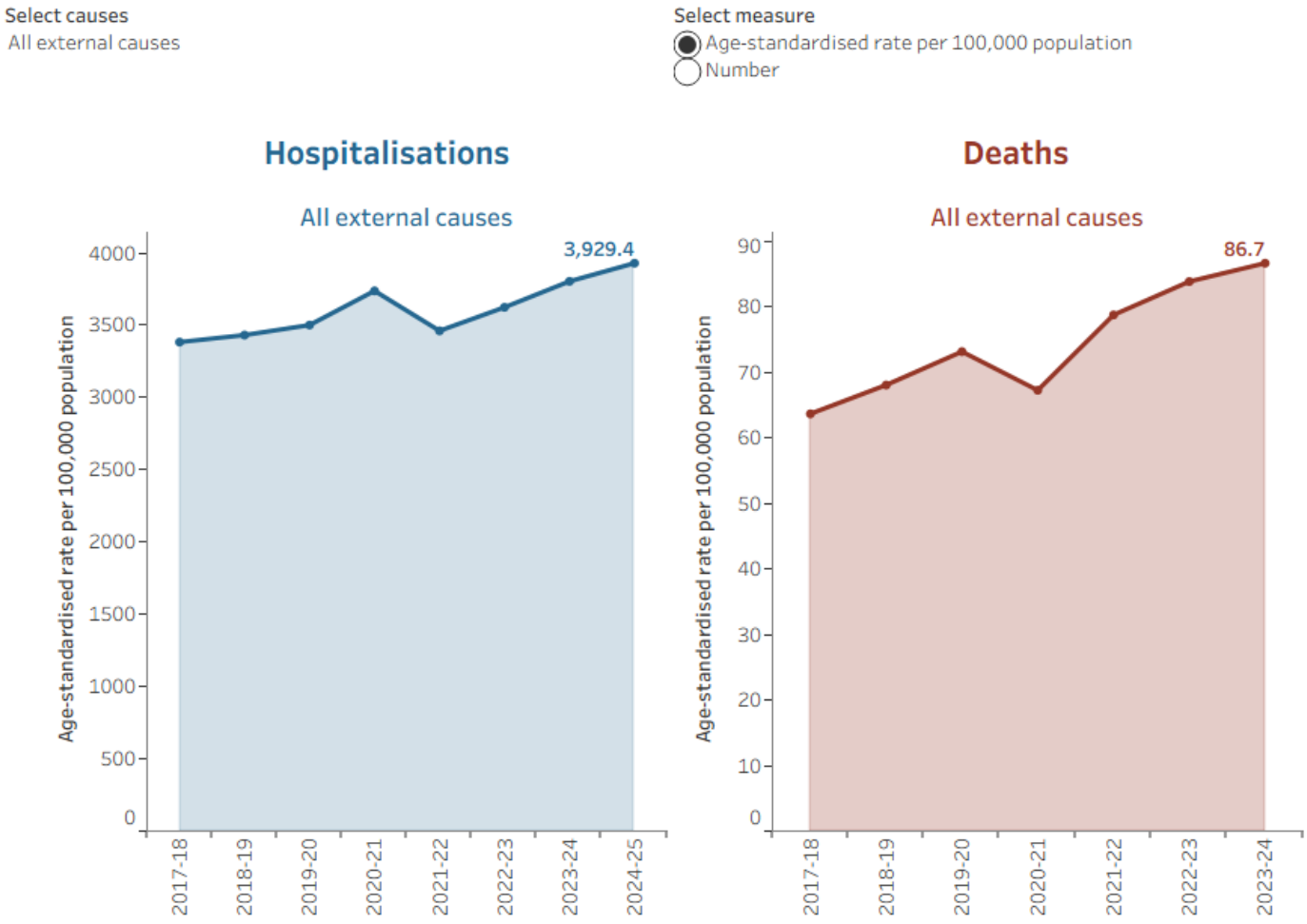
1. Deaths data only includes data for New South Wales, Queensland, Western Australia, South Australia, and the Northern Territory, as these jurisdictions are considered to have adequate levels of Indigenous identification in mortality data.
2. The sum of the counts by cause may be greater than the total number of injury deaths because some deaths have multiple causes.
3. Mortality rates are crude per 100,000 population.

Sources: AIHW National Mortality Database and ABS Estimates and Projections, Aboriginal and Torres Strait Islander Australians.

Trends by cause

The age-standardised rate (ASR) of injury hospitalisations among First Nations people has steadily increased from 3,501.2 per 100,000 (representing 30,550 cases) in 2019–20 to 3,929.4 per 100,000 in 2024–25 (37,730 cases) (Figure 23). The most recent rate is an 8.4% increase compared with the average rate of 3,625.7 reported over the 5-year period from 2019–20 to 2023–24.

Figure 23: Trends in injury hospitalisations (2017–18 to 2024–25) and deaths (2017–28 to 2023–24) among First Nations people by cause



Notes:

1. Deaths data only includes data for New South Wales, Queensland, Western Australia, South Australia, and the Northern Territory, as these jurisdictions are considered to have adequate levels of Indigenous identification in mortality data.
2. Age-standardised rates per 100,000 population.

Sources: AIHW National Hospital Morbidity Database, AIHW National Mortality Database, and ABS Estimates and Projections, Aboriginal and Torres Strait Islander Australians.

The largest change in injury hospitalisations by cause (based on age-standardised rates) compared to the previous 5-year average was observed for *Electricity and air pressure* (a 51.7% increase, from 2.9 to 4.4 per 100,000 population in 2024–25). Other notable changes include:



Contact with living things

Injury hospitalisations in 2023–24 increased by **16.4%**, to an ASR of 245.6 per 100,000 population

(compared to the previous 5-year average)



Falls

Injury hospitalisations in 2023–24 increased by **12.9%**, to an ASR of 1,079.7 per 100,000 population

(compared to the previous 5-year average)



Thermal causes

Injury hospitalisations in 2023–24 decreased by **11.7%**, to an ASR of 49.7 per 100,000 population

(compared to the previous 5-year average)



Intentional self-harm

Injury hospitalisations in 2023–24 decreased by **11.6%**, to an ASR of 256.4 per 100,000

(compared to the previous 5-year average)

In 2023–24, the age-standardised injury death rate among First Nations people rose by 11.2% to 86.7 per 100,000 (701 deaths) compared to the 2018–19 to 2022–23 average.

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Comparison

Comparison between First Nations people and non-Indigenous Australians

Injury ED presentations

First Nations people in 2023–24 were **2.1 times** as likely to present to an Emergency Department (ED) for injuries than non-Indigenous people.

Injury hospitalisations

First Nations people in 2023–24 were **2 times** as likely to be hospitalised due to injuries as non-Indigenous people.

Injury deaths

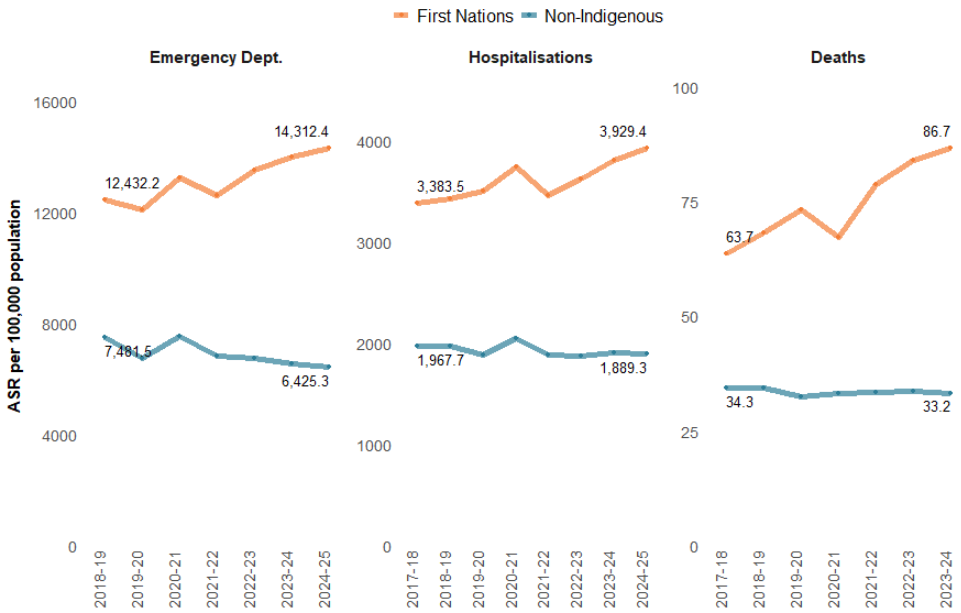
First Nations people in 2022–23 were **2.8 times** as likely to die from an injury as non-Indigenous people.

The gap in ED presentations between First Nations and non-Indigenous Australians has expanded over the last 6 years (Figure 24).

Age-standardised rates (ASRs) among First Nations people increased from 12,432.2 to 14,312.4 ED presentations per 100,000 population, while rates for non-Indigenous Australians declined from 7,481.5 to 6,425.3 per 100,000 population during the same time period. The Non-Admitted Patient Emergency Department Care (NAPEDC) Database provides information on ED presentations, but does not contain information about the causes behind this gap.

Similarly, the gap in injury hospitalisations has widened over the last decade (Figure 24), increasing from 1.7 to 2.1 times higher between 2017-18 and 2024-25.

Figure 24: Injury ED presentations (2018–19 to 2024–25), hospitalisations (2017–18 to 2024–25) and deaths (2017–18 to 2023–24) by Indigenous status



Notes:

1. Age-standardised rates (ASR) per 100,000 population
2. Deaths data only includes data for New South Wales, Queensland, Western Australia, South Australia, and the Northern Territory, as these jurisdictions are considered to have adequate levels of Indigenous identification in mortality data.

Sources: AIHW National Hospital Morbidity Database, AIHW National Mortality Database, AIHW National Non-admitted Patient Emergency Department Care Database and ABS Estimates and Projections, Aboriginal and Torres Strait Islander Australians.

For individual causes of injuries, First Nations people had a higher rate of hospitalisation than the non-Indigenous population due to:

- assault (around 784.4 cases, or 55.5 per 100,000) or 14.1 times as likely as non-Indigenous people
- intentional self-harm (around 257.8 cases, or 76.7 per 100,000) or 3.4 times as likely
- thermal causes (around 59.5 cases, or 19 per 100,000) or 3.1 times as likely.

First Nations people had higher injury death rates than non-Indigenous Australians for most causes, being about 7 times as likely to die from homicide (Table 7).

Table 7: Ranked rate ratio of death among First Nations people compared to non-Indigenous Australians by cause of injury, 2023–24

External cause of injury	First Nations ASR	Non-Indigenous ASR	Rate Ratio
Homicide	4.2	0.6	7.0
Accidental poisoning	18.9	3.4	5.6
Transport	12.9	3.5	3.7
Suicide	26.0	8.0	3.3
All external causes	**86.7**	**33.2**	**2.6**
Choking and suffocation	6.5	2.9	2.2
Falls	13.4	12.4	1.1

Notes:

1. Age-standardised rates (ASRs) per 100,000 population.
2. External causes with ASR numerators under 20 are not shown.

Sources: AIHW National Mortality Database and ABS Estimates and Projections, Aboriginal and Torres Strait Islander Australians.

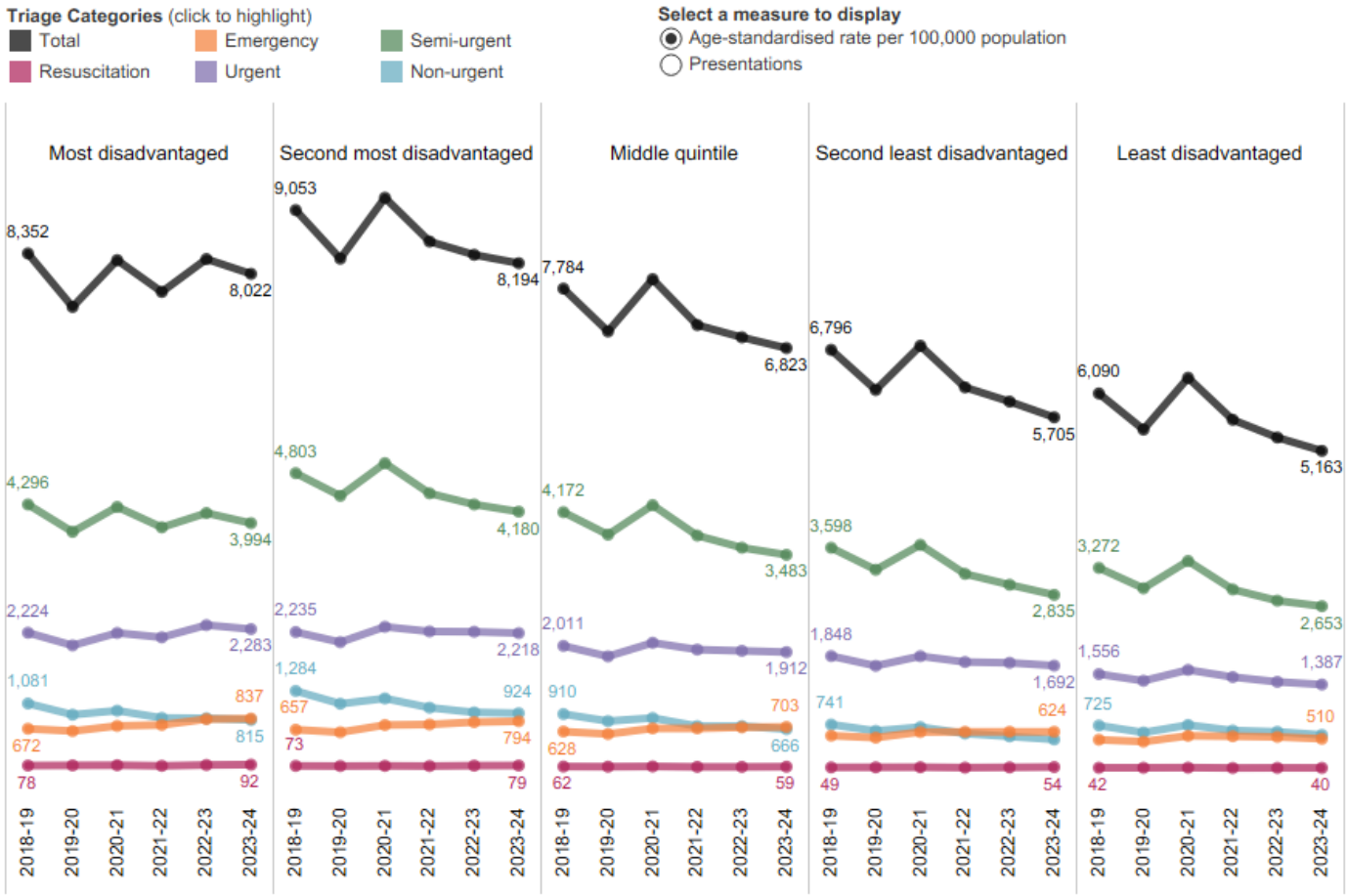
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Socioeconomic areas

People living in the most disadvantaged socioeconomic areas tend to present to an emergency department (ED) with injuries more frequently than those living in other areas (Figure 25).

When compared with people living in the least socioeconomically disadvantaged areas, those living in the most disadvantaged areas were 1.6 times as likely to present to ED for injury in 2023–24.

Figure 25: Number and age-standardised rate of ED presentations for injury, by socioeconomic status, Australia, 2018–19 to 2023–24



Notes:

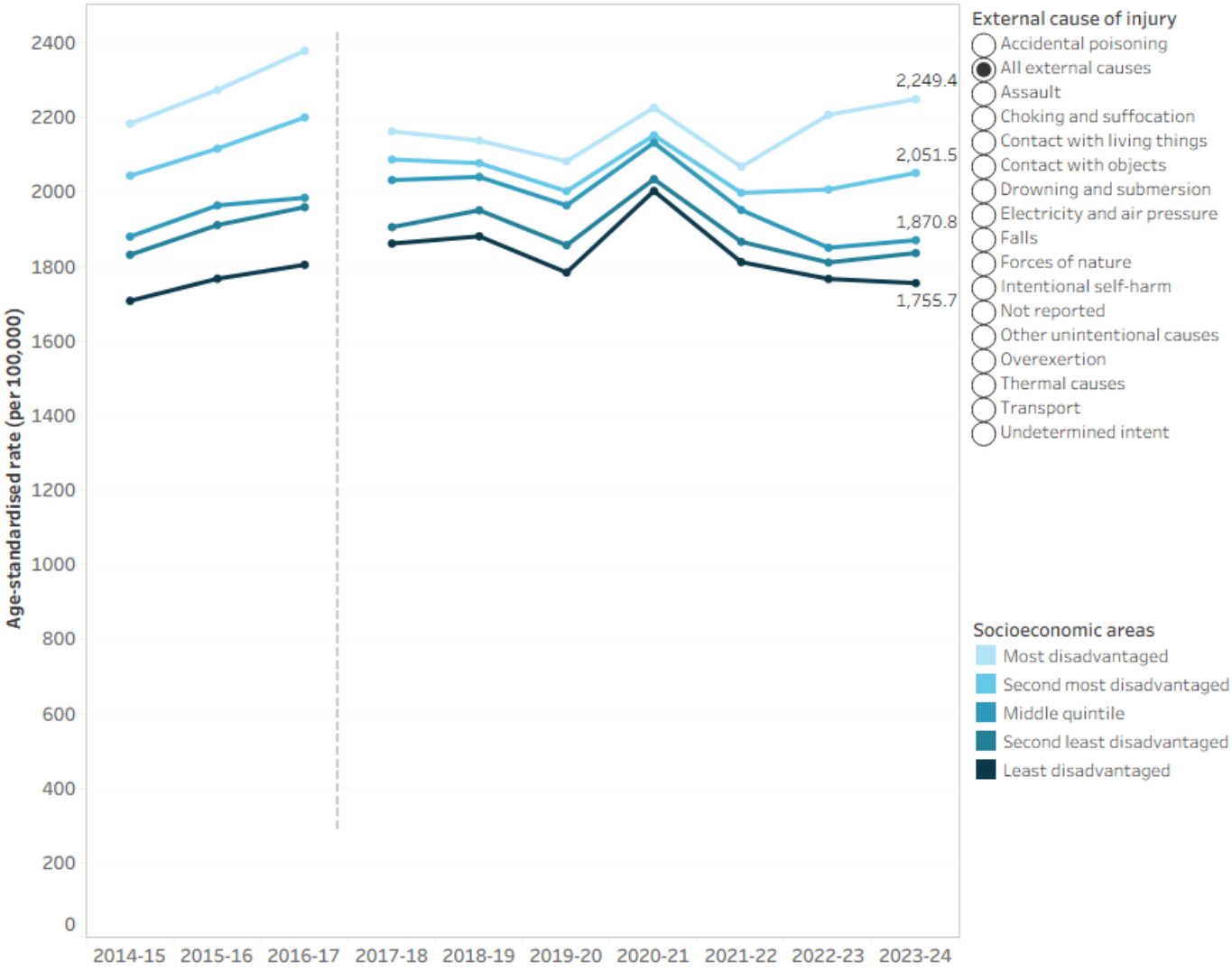
1. Age-standardised rates per 100,000 population.
2. See Severity for more information on triage categories.

Sources: AIHW National Non-admitted Patient Emergency Department Care Database, and ABS National, state and territory population.

In 2023–24, both the number and rate of injury hospitalisations were highest among the most socioeconomically disadvantaged area (121,202 cases, 2,249.4 per 100,000 people). The number of injury hospitalisations decreased with affluence to 101,743 cases in the least disadvantaged area and an age-standardised rate of 1,755.7 per 100,000 people (Figure 26).

For deaths in 2022–23, there was a clear trend with rates increasing with disadvantage. The number and rate were highest among those most disadvantaged (3,611 cases, an age-standardised mortality rate of 61.7 per 100,000 people), decreasing to 2,415 cases in the least disadvantaged quintile of the population (an age-standardised rate of 37.2 per 100,000 people).

Figure 26: Age-standardised rates of injury hospitalisations by socioeconomic areas, 2014–15 to 2023–24, Australia



Note: Age-standardised rates per 100,000 population.

Sources: AIHW National Hospital Morbidity Database and ABS National, state and territory population.

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Geography

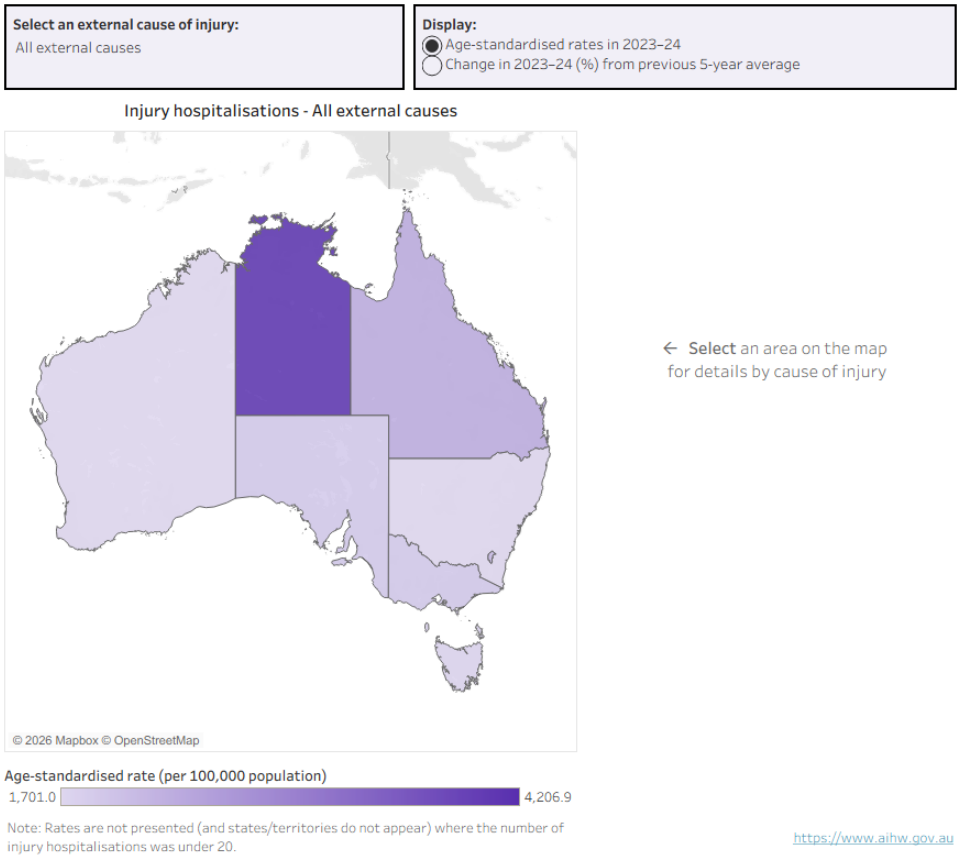
Injury hospitalisations and external causes of injury vary by location.

The dashboard below presents injury hospitalisation rates across Australia at two sub-national geographic levels: States and Territories, and Statistical Areas 3 (SA3).

At the SA3 geographic level, age-standardised rates (ASRs) are calculated by aggregating data across 3 years so that information for smaller regions can be reliably displayed (see the Notes tab for details).

The dashboard allows you to select specific external causes of injury, and offers two metrics for display:

- The most recent ASR values (2023–24 at the State and Territory level, 2021–22 to 2023–24 at the SA3 level)
- The relative change in ASR in 2023–24 compared to an average of the previous 5 financial years



This interactive visualisation present age-standardised rates of injury hospitalisations by external cause of injury at the State and Territory level, and at the SA3 level.

For technical details about the data, see the technical notes.

Geographic influences are also reflected in the remoteness data sections discussed in the next page.

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Remoteness

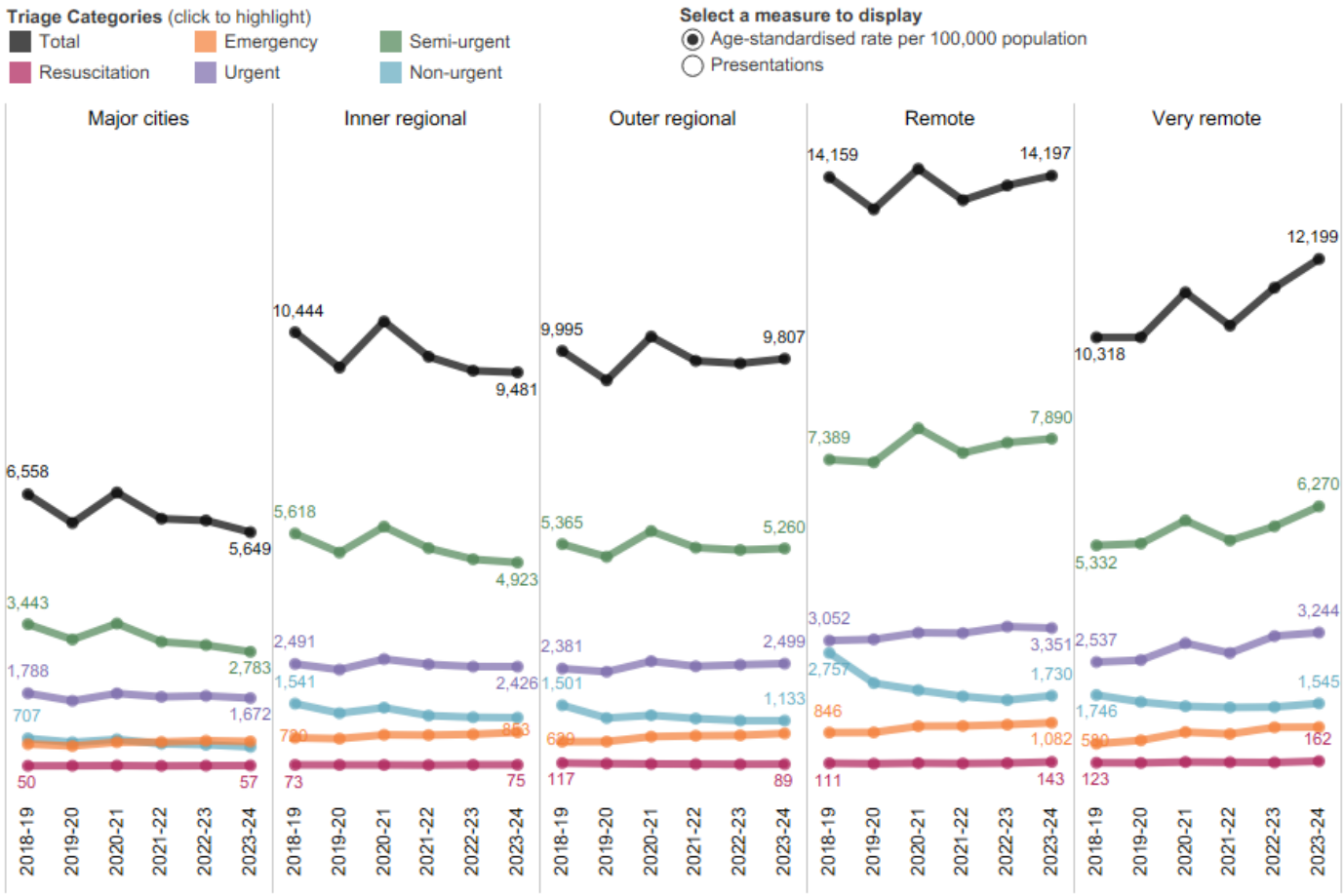
Remoteness of the area of residence impacts the risk of injuries. In general, residents of rural or more disadvantaged areas experience higher rates of injury hospitalisation and death compared to people who live in urban areas (Mitchell and Chong, 2010).

People living in *Remote* areas, when compared with people living in *Major cities*, were 2.5 times as likely to present to an Emergency Department (ED) for an injury in 2023–24.

Over the last 6 years, people living in *Remote* areas were the most likely to present to ED with an injury, while those in *Major cities* were the least likely (Figure 27).

The NAPEDC Database describes ED presentations, but does not contain information about the reasons for the differences in presentations between remoteness areas.

Figure 27: Number and age-standardised rate of injury ED presentations, by remoteness and triage category, Australia, 2018–19 to 2023–24



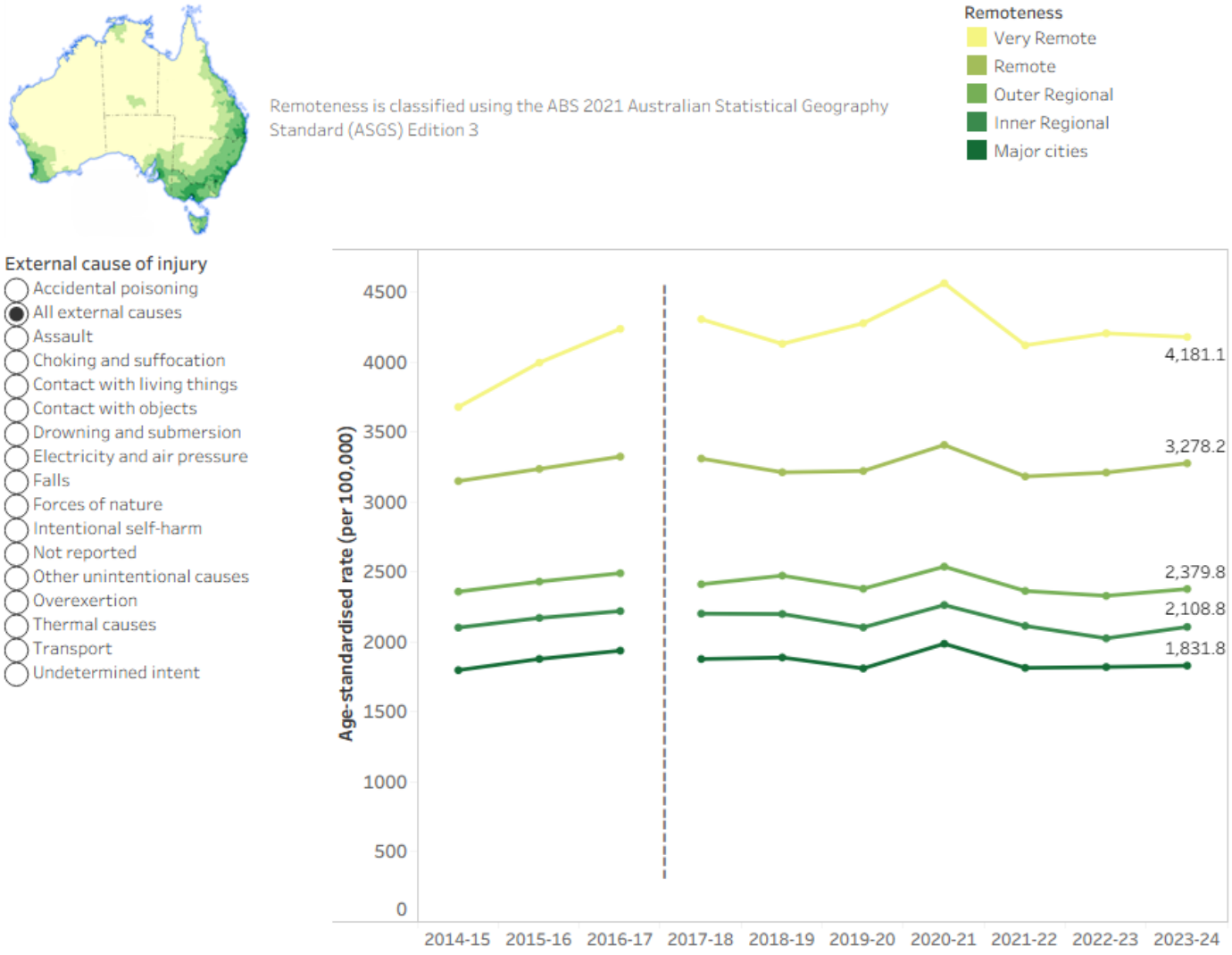
Notes:

1. Age-standardised rates per 100,000 population.
2. Remoteness regions are classified using the ABS 2021 Australian Statistical Geography (ASGS) Edition 3. Rates are updated when the relevant denominator data becomes available from the ABS.
3. See Severity for more information on the triage categories.

Sources: AIHW National Non-admitted Patient Emergency Department Care Database, and ABS National, state and territory population.

In 2023–24, injury hospitalisation rates indicated *Major cities* dwellers were the least likely to be hospitalised due to injury across regions (381,860 cases, with an age-standardised rate of 1,831.8 per 100,000 people). Age-standardised rates of injury hospitalisation increased with remoteness, with the highest rate per resident population observed in *Very remote* areas (4,181.1 per 100,000 persons) (Figure 28).

Figure 28: Age-standardised rates of injury hospitalisations by remoteness, Australia, 2014–15 to 2023–24



Note: Age-standardised rates per 100,000 population.

Sources: AIHW National Hospital Morbidity Database, and ABS National, state and territory population.

Reference

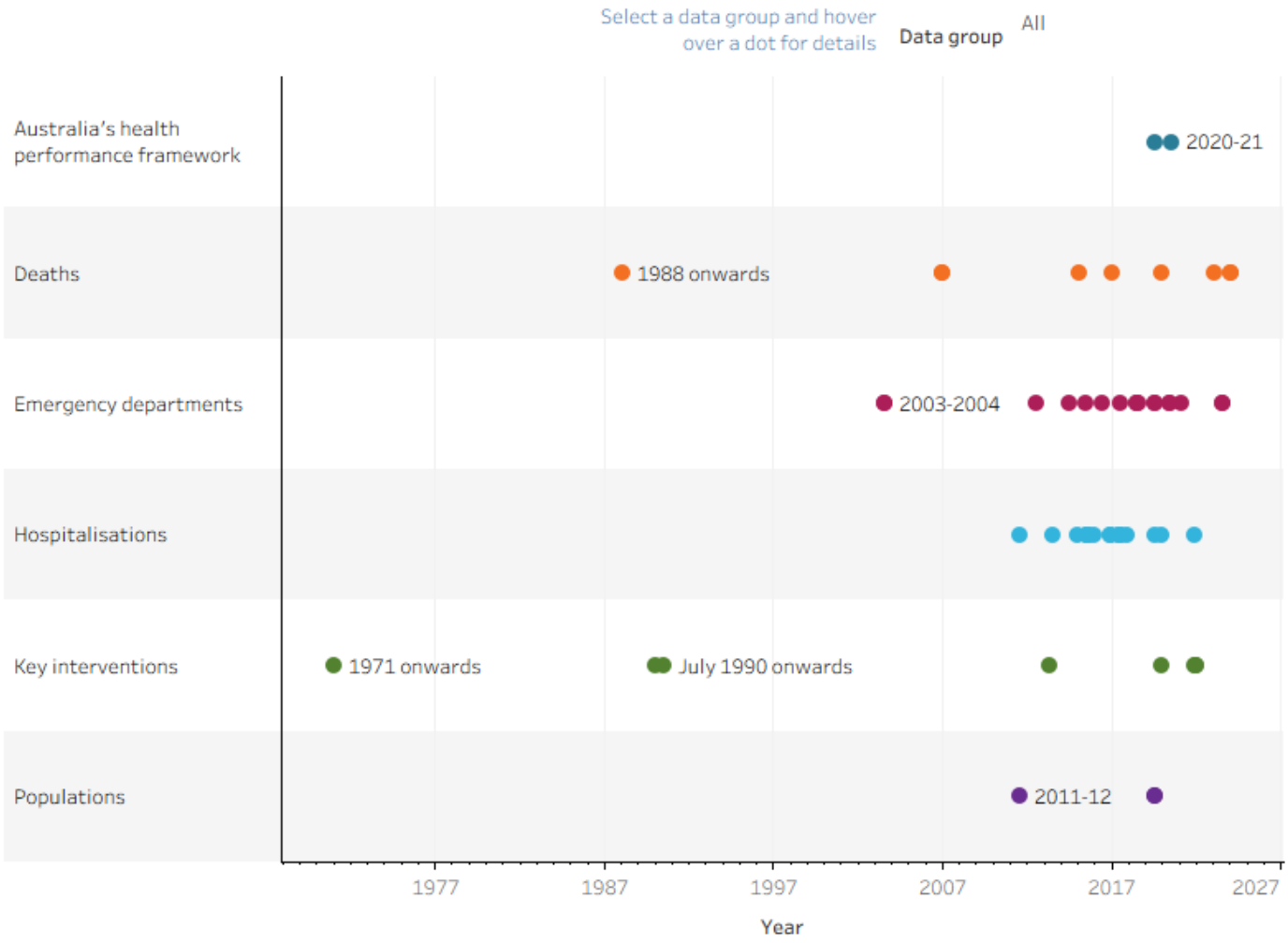
Mitchell RJ & Chong S (2010) *Comparison of injury-related hospitalised morbidity and mortality in urban and rural areas in Australia*. Rural and Remote Health 10:123-33.

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Technical notes

Injury surveillance data has changed over time. This visualisation shows the timeline of key changes in injury data sources from 1971 (Figure T1).

Figure T1: Timeline of injury surveillance data, Australia, 1971–2026



Notes:

1. Where the change description is '*' there are multiple changes on a date. Select that data group to expand and display changes.
2. Detailed technical notes about hospitals and emergency department data can be accessed via AIHW's Hospitals About the data.
3. Detailed notes about deaths data can be accessed via National Mortality Database information.

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Data sources

The AIHW operates under a strict privacy regime which has its basis in Section 29 of the *Australian Institute of Health and Welfare Act 1987* (AIHW Act). Section 29 requires that confidentiality of data relating to persons (living and deceased) and organisations be maintained. The *Privacy Act* governs confidentiality of information about living individuals. The AIHW is committed to reporting that maximises the value of information released for users while being statistically reliable and meeting legislative requirements described in the AIHW Act and the *Privacy Act*.

Hospitalisations

Hospitalisations data are sourced from the Australian Institute of Health and Welfare's (AIHW) National Hospital Morbidity Database (NHMD). The NHMD is a compilation of episode-level records from admitted patient morbidity data collection systems (APC NMDS) in Australian public and private hospitals. It includes episodes of care for admitted patients in all public and private acute and psychiatric hospitals, free standing day hospital facilities and alcohol and drug treatment centres in Australia. Hospitals operated by the Australian Defence Force, corrections authorities and in Australia's offshore territories may also be included. Hospitals specialising in dental, ophthalmic aids and other specialised acute medical or surgical care are included. Data quality statements for the NHMD are available on AIHW Hospitals. For more information about data contained in the NHMD refer to the AIHW Hospitals technical notes.

Emergency department presentations

Emergency department (ED) data are sourced from the National Non-admitted Patient Emergency Department Care Database (NNAPEDCD). Data quality statements for this dataset are available on the AIHW Hospitals website. For the 2024-25 NAPEDC National Minimum Data Set (NMDS)/ National Best Endeavours Data Set (NBEDS), diagnosis information was reported using the ED ICD-10-AM version 12 shortlist that can be found on the website of the Independent Hospital Pricing Authority.

For more information about data contained in the NNAPEDCD refer to the Hospitals technical notes for recent years.

Deaths

Deaths data are sourced from the AIHW National Mortality Database (NMD). When a person is declared dead, information about their death is recorded on a death certificate by either a medical practitioner or coroner. Registration of all deaths is compulsory in Australia and is the responsibility of the Registrar of Births, Deaths and Marriages of the relevant state or territory, under jurisdiction-specific legislation.

Deaths data are assembled, coded, and published on behalf of the Registrars by statistical agencies. These agencies have varied since 1900 and have included state-based offices and what is now the Australian Bureau of Statistics (ABS). Information is also provided to the ABS via the National Coronial Information System (NCIS) for those deaths certified by a coroner.

The ABS codes causes of death according to the International statistical classification of diseases and related health problems, 10th revision (ICD-10) (WHO 2019) and, after checks and de-identification, creates the Cause of Death Unit Record File (CODURF).

The CODURF contains characteristics of the person who died (for example, age, sex, and Indigenous status), and characteristics of their death (for example, causes of death, date, and place where the person usually lived). The AIHW maintains these data in the NMD.

The data quality statements underpinning the AIHW NMD can be found on the following ABS web pages:

- Australian Bureau of Statistics (2025), Deaths, Australia methodology
- Australian Bureau of Statistics (2025), Causes of Death, Australia methodology

For more information on mortality coding refer to Causes of Death, Australia Methodology, 2024.

Population data

Population data are used for demographic analyses and as the denominator in calculating rates. All population level calculations are based on the estimated resident population (ERP) calculated as at the midpoint of each financial year. For example, for the reporting period 2024–25, the denominator population is the June 2024 ERP + the June 2025 ERP, divided by 2. This is used as the denominator for age-specific/crude and age-standardised rates.

The ERP as at 30 June 2001 is used as the standardising population throughout the report (ABS 2003).

All population data are sourced from the Australian Bureau of Statistics (ABS) as follows:

- General populations are from National, state and territory population (ABS 2025)
- First Nations populations are from Estimates and Projections, Aboriginal and Torres Strait Islander Australians (ABS 2024)
- Remoteness populations are from Regional populations by age and sex (ABS 2025)
- Socio-Economic Indexes for Areas (SEIFA) Index of Relative Socio-Economic Disadvantage (IRSD) quintile populations are from AIHW analysis of Socio-Economic Indexes for Areas (SEIFA), Australia (ABS 2021)

First Nations people & non-Indigenous Australians

Since estimates of Aboriginal and Torres Strait Islander (First Nations) populations are only provided for 30 June, estimates for 31 December are calculated by adding 2 consecutive 30 June estimates and dividing by 2 (for example, the estimate for 31 December 2024 is calculated by adding estimates for 30 June 2024 and 30 June 2025 then dividing by 2).

For non-Indigenous Australians, population denominators were derived by subtracting the estimated First Nations population from the Australian national estimated resident population, as at 31 December of the relevant year.

Notes on data

The following challenges apply to the data sources utilised in the *Injury in Australia* report:

1. All data informing this report are sourced from administrative data collected for the primary purpose of health care provision within jurisdictions. Analyses of these data to inform the *Injury in Australia* report is a secondary use and as such, data may be missing or unavailable in the detail necessary to answer specific questions. This may be because information was not reported by patients, was not recorded in the clinical notes or was not coded into administrative records. Additionally, cause of death records contain only summary data, not detailed information around the circumstances of death, and definitive diagnoses informing the cause death may not be available from the data sourced during emergency department (ED) presentations or episodes of care prior to death occurring.
2. All data sources informing this report are primarily quantitative in nature, therefore *Injury in Australia* presents descriptive analyses of patterns and trends but cannot address qualitative questions around why certain patterns or trends are occurring.
3. *Injury in Australia* does not currently use linked data sources and cannot identify the numbers of people injured within any data source. Individuals can experience multiple injuries and present to multiple health care facilities within any time period. The methods described in the technical notes count cases of injury in order to obtain estimated injury incidence.
4. External cause data (such as place, mechanism or intent) is fundamental for understanding the causes and circumstances of injury presentations to emergency departments (EDs). Jurisdictions do collect information about the external cause and circumstances of injury, but there is currently no national standard for this data, and free text fields containing this data are currently unavailable for analyses. External cause information is therefore unavailable for injury related ED presentations.
5. The data used in this report may be retrospectively updated by jurisdictions as records are updated, and timeseries data may therefore alter over time.
6. Cause of death data becomes available to users for analysis following cause of death coding, validation and release by the ABS and approval from the Australian Coordinating Registrar. As a result, injury death reporting lags behind hospitalisation and ED reporting by 1 financial year.
7. Reported data may compare groups that vary widely in case count, largely due to differences in population size (for example, the population of New South Wales is more than 30 times as large as the Northern Territory population, and the population of *Major cities* is nearly 90 times that of *Very remote* areas). This means that year-to-year changes in counts or rates for the smaller-population groups might be subject to large random variation and such fluctuations could potentially be misinterpreted as meaningful rises or falls.

States and territories are primarily responsible for the quality of the data they provide. However, the AIHW undertakes extensive validations on receipt of data. Potential errors are queried with jurisdictions, and corrections and resubmissions may be made in response to these queries. The AIHW does not adjust data to account for possible data errors or missing values, except where stated. The AIHW takes active steps to improve the consistency of these data over time.

References

ABS (Australian Bureau of Statistics) (2003) Population by age and sex, Australian states and territories, 2001: Census edition final. ABS cat. no. 3201.0. Canberra: ABS.

ABS (2024) Estimates and projections, Aboriginal and Torres Strait Islander Australians, 2006 – 2031, ABS website, Australian Government, accessed 4 June 2025.

ABS (2021) Socio-Economic Indexes for Areas (SEIFA), Australia, ABS website, accessed 10 May 2024.

ABS (September 2023), National, state and territory population, ABS website, accessed 10 May 2024.

WHO (World Health Organization) (2019) ICD-10: international statistical classification of diseases and related health problems: tenth revision, WHO, Geneva.

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Injury Emergency Department presentations

Emergency Department (ED) care is a form of non-admitted hospital care. While ED presentation records have many of the same fields as hospital records, there are key differences in the way they are structured.

A notable difference in ED records compared with hospitalisations and deaths, is the lack of external cause data. External cause and related data on the injury include details such as place of occurrence, mechanism of the injury, activity being undertaken at the time, intent and perpetrator. Additionally, there are unique variables only found within ED records such as triage category and waiting time that cannot be compared against hospitalisations and deaths.

ED presentations for injury incidents are generally more numerous than the number of hospitalisations. This is because many injuries can be treated in EDs and do not require admission to a hospital. Many more people with injuries are treated outside of a hospital such as at a general practice – these injuries are not captured in ED data.

For summary of key data quality issues affecting ED data, please refer to the interactive timeline in Figure T1.

Injury case identification

An ED presentation for an injury is defined where the following apply:

1. An ED record is present with a principal diagnosis of ICD-10-AM codes in the range S00–T75 or T79. In 2023–24, about 92% of injury related ED records had the injury recorded as principal diagnosis for presentation, about 8% had an additional diagnosis of injury recorded, with or without a principal diagnosis of injury.
2. The physical departure date and time are between midnight of 1 July and midnight of 30 June of the financial year of reporting.
3. A person may have more than one injury recorded during a presentation, if both principal and additional diagnoses include injuries, that presentation is counted once.
4. Cases that are hospitalised after presenting to EDs are counted in both ED and hospitals data sources.
5. A small number of severe injuries result in the person being dead on arrival at the ED, these cases are counted in both the ED and deaths data sources.

This scope excludes injuries due to *Complications of surgical and medical care* (T80–T88) and *Sequelae of injuries, of poisoning and of other consequences of external causes* (T90–T98) in line with our reporting on injury hospitalisations.

Definitions and classifications

If not otherwise indicated, data elements were defined according to their definitions in the AIHW's Metadata Online Registry (METEOR) and summarised in the Glossary.

In particular, data element definitions for the Non-admitted patient ED care National Minimum Data Set (NMDS) are available online at: METEOR website.

External cause data

The National Non-admitted Patient Emergency Department Care Database (NNAPEDCD) does not include a field for external cause of injury (such as a fall or transport accident) or other related data such as place of occurrence, mechanism of the injury, activity being undertaken at the time, intent and perpetrator. Australian injury surveillance systems have a major focus on the external causes of injuries, which is especially important from a prevention perspective. The absence of these national data obstructs direct comparisons between the causes of injury across hospitalisations, deaths, and ED presentation data.

Reporting diagnosis information

For the 2024–25 NAPEDC NMDS/NBEDS, diagnosis information was reported using the ED ICD-10-AM version 12 shortlist that can be found on the website of the Independent Hospital Pricing Authority. From 2020–21 to 2022–23, the short list was based on ICD-10-AM version 11.

Episode end status

There is a difference between the number of presentations with a type of visit of *Dead on arrival* and the number of presentations with an episode end status of *Dead on arrival*. All presentations with a type of visit of *Dead on arrival* had an episode end status of *Dead on arrival*. However, some presentations with an episode end status of *Dead on arrival* did not have a type of visit of *Dead on arrival*.

Waiting time to commencement of clinical care

The waiting times are determined as the time elapsed between presentation to the ED and the commencement of clinical care. The calculation is restricted to presentations with a type of visit of *Emergency presentation*, and presentations were excluded if the waiting time was missing or invalid, or if the patient *Did not wait to be attended by a health care professional,* was *Dead on arrival* or *Registered, advised of another health service and left without being attended to*.

Proportion of presentations seen on time

The proportion of presentations seen on time was determined as the proportion of presentations in each triage category with a waiting time less than or equal to the maximum waiting time stated in the Australasian Triage Scale definition. Triage categories and respective clinically appropriate waiting times are described in Table T1 (for further details see AIHW METEOR).

Table T1: Clinically appropriate waiting times by category of ED presentation

Category	Clinically appropriate waiting time
1: Resuscitation	Immediate (within seconds)
2: Emergency	Within 10 minutes
3: Urgent	Within 30 minutes
4: Semi-urgent	Within 60 minutes
5: Non-urgent	Within 120 minutes

For *Injury in Australia*, a patient with a triage category of Resuscitation is considered to be seen on time if the waiting time to commencement of clinical care is less than or equal to 2 minutes.

The calculation of waiting times is restricted to presentations with a type of visit of *Emergency presentation*, and presentations were excluded if the waiting time was missing or invalid, if the patient *Did not wait to be attended by a health care professional,* or was *Dead on arrival*, or if the triage category was not reported.

Quality of data on ED waiting times

The criteria used to determine the proportion of *Resuscitation* patients seen on time varies between jurisdictions, therefore, the proportions of *Resuscitation* patients seen on time may differ from those reported by individual jurisdictions.

Proportion of presentations ending in admission

The proportion of presentations ending in admission is determined as the proportion of all emergency presentations with an episode end status of *Admitted to this hospital* (either short-stay unit, hospital-in-the-home, or non-ED hospital ward).

Analysis methods

The Australian ERP as at 30 June 2001 is used as the standardising population throughout the report. Age-standardisation of rates enables valid comparison across years and/or jurisdictions without being affected by differences in age distributions.

Population-based rates of injury tend to have similar values from one year to the next. Exceptions to this can occur (for example, due to a mass-casualty disaster), but are unusual in Australian injury data. Some year-on-year variation and short-run fluctuations are to be expected, so small changes in a rate over a short period do not provide a firm basis for asserting that a trend is present.

All rate calculations (Table T2) utilise a denominator based on the estimated resident population (ERP) calculated as at the midpoint of each financial year. For example, for the reporting period 2024–25, the denominator population is the June 2024 ERP + the June 2025 ERP, divided by 2. This is used as the denominator for age-specific/crude and age-standardised rates. Rates are calculated for each financial year unless otherwise noted.

Table T2: Calculation methods for measures of ED presentations related to injury in Australia

Measure	Numerator	Denominator	Calculation
Population (used for rates)	June 2024 population + June 2025 population	2	Numerator ÷ Denominator
Crude or age-specific rate	Number of injury ED presentations per defined category (e.g. age group)	Estimated Australian population as at mid-point of financial year	(Numerator ÷ Denominator) x 100,000
Age-standardised rate (ASR). ASRs were derived using 5-year age groups up to 85+. ASRs for First Nation populations were derived using 5-year age groups up to 65+.	Expected events per age group in standard population= crude rate x standard population (for each corresponding age group)	n.a.	The direct method of standardisation is used. (Sum of numerators across all age groups ÷ total standard population) x 100,000
Change in rates	n.a.	n.a.	Estimated trends in age-standardised rates were reported as average annual percentage changes.

Median waiting time			The 50th percentile (the median, or the middle value in a group of data arranged from lowest to highest value for minutes waited) represents the number of minutes within which 50% of patients commenced clinical care (or completed their episode or were admitted) – half the waiting times will have been shorter, and half longer than the median.
90th percentile waiting time			The 90th percentile data represent the number of minutes (or hours and minutes) within which 90% of patients commenced clinical care (or completed their episode or were admitted).
Waiting times			The waiting times are determined as the time elapsed between presentation to the emergency department and the commencement of clinical care. The calculation is restricted to presentations with a type of visit of *Emergency presentation*, and presentations were excluded if the waiting time was missing or invalid, or if the patient *Did not wait to be attended by a health care professional* or was *Dead on arrival*.
Proportion of presentations seen on time			The proportion of presentations seen on time was determined as the proportion of presentations in each triage category with a waiting time less than or equal to the maximum waiting time stated in the Australasian Triage Scale definition. Triage categories and respective clinically appropriate waiting times are described in Table 1 (for further details see AIHW METEOR). For Injury in Australia, a patient with a triage category of Resuscitation is considered to be seen on time if the waiting time to commencement of clinical care is less than or equal to 2 minutes.

n.a. Not Applicable

Median and 90th percentiles

The 50th percentile and 90th percentile waiting times are calculated using an empirical distribution function with averaging. Using this method, observations are sorted in ascending order.

The calculation is where:

n is the number of observations,

and *p* is the percentile value divided by 100,

then $n \times p = i + f$ (where *i* is an integer and *f* is the fractional part of $n \times p$).

If $n \times p$ is an integer, the percentile value will correspond to the average of the values for the *i* th and (*i+1*)th observations.

If $n \times p$ is not an integer, the percentile value will correspond to the value for the (*i+1*)th observation.

For example, if there were 100 observations, the median waiting time will correspond to the average waiting time for the 50th and 51st observations (ordered according to waiting time). Similarly, the 90th percentile will correspond to the average waiting time for the 90th and 91st observations if there are 100 observations.

If there were 101 observations, the median waiting time will correspond to the waiting time for the 51st observation, and the 90th percentile waiting time will correspond to the waiting time for the 91st observation.

The 50th and 90th percentiles have been rounded to the nearest whole number of minutes.

Timeseries

Time series rates may not match historically published rates due to changes in denominator data over time, jurisdictional updates in numerator data or retrospective refreshing of numerator or denominator data across different data sources.

Presentation of data

An ED presentation may have more than one recorded injury, each presentation has been counted once if it has multiple injuries recorded.

Persons totals include presentations for which sex was not reported.

All age totals include presentations where age was not reported.

Crude/age-specific rates and age-standardised rates are calculated per 100,000 population and are rounded to one decimal place (for example, 3.4 per 100,000).

Data suppression and confidentiality

Data (cells) in tables may be suppressed to maintain the privacy or confidentiality of a person or organisation; or because a proportion, rate (numerator or denominator) or other measure is related to a small number of events (and may therefore not be reliable). Data may also be suppressed to avoid attribute disclosure. The abbreviation 'n.p.' (not published) has been used in tables to denote these suppressions. In these tables, the suppressed information is included in the totals.

Counts

Counts of under 5 are suppressed and consequential suppression is applied.

Crude rates

- Crude rates with counts (numerator for calculation) less than 10 are suppressed.
- If the corresponding counts measure is suppressed, the crude rate has been suppressed.

Age-standardised rates

- Age-standardised rates with counts (numerator for calculation) less than 20 are suppressed.
- If the corresponding counts measure is suppressed, the age-standardised rate has been suppressed.

Z-score

No suppression applied.

Data quality

Overall, the quality of the data in the NNAPEDCD is sufficient to be published in this report. However, the following limitations of the data should be taken into consideration when data are interpreted.

States and territories are primarily responsible for the quality of the data they provide. However, the AIHW undertakes extensive validations on receipt of data. Potential errors are queried with jurisdictions, and corrections and resubmissions may be made in response to these edit queries. The AIHW does not adjust data to account for possible data errors or missing values, except where stated.

The AIHW takes active steps to improve the consistency of these data over time.

For 2022–23, the NNAPEDCD may not include emergency presentations to hospitals that have emergency departments that are not in scope for the NAPEDC NMDS. The inclusion criteria for emergency departments may exclude some smaller regional public hospitals.

Prior to 2020–21, the following jurisdictions have provided data to the NNAPEDCD using the NAPEDC National Best Endeavours Data Set (NBEDS) specification:

- Queensland (from 2015–16 to 2019–20)
- Victoria and Western Australia (from 2016–17 to 2019–20).

All other states and territories used the NAPEDC NMDS. The data provided using the NAPEDC NBEDS may not be entirely comparable with data provided using the NAPEDC NMDS.

Although there are national standards for data on non-admitted patient emergency department services, the way those services are defined and counted varies across states and territories, and over time.

Please also refer to the interactive timeline in Figure T1 for details of changes in ED data over time.

Missing or invalid data

In some cases, the data provided may include missing values (for example, the date/time of physical departure was not recorded), or invalid values (for example, if the time of physical departure was recorded as occurring before the time of presentation).

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Injury hospitalisations

Injury case identification

A diagnosis of injury is defined as ICD-10-AM codes in the range S00–T75 or T79, using 'Chapter 19 Injury, poisoning and certain other consequences of external causes'. A primary diagnosis of injury is when one of the specified codes is the first diagnosis code reported, while an additional diagnosis of injury is when one of the specified codes is reported but not as the first diagnosis.

A person may have more than one incident of injury resulting in hospitalisation in a financial year and each case of hospitalisation will be counted separately in this report. This is because we are counting incidents of injury resulting in hospitalisation, rather than the number of people who were hospitalised, in a given financial year. If a single incident led to an admission in more than one hospital, the incident has only been counted once. Therefore, counts of injury cases will be lower than the count of hospital records indicating injuries.

Inclusion criteria

- Records with the maximal snapshot ID in any database where the date of separation falls within the timeframe defined in the report.
- National Hospital Morbidity Database (NHMD) records with a principal diagnosis in the ICD-10-AM range S00–T75 or T79, using *Chapter 19 Injury, poisoning and certain other consequences of external causes*.
- NHMD records with a separation date between 1 July 2024 and 30 June 2025.

Exclusion criteria

- Records were excluded where the Australian Institute of Health and Welfare (AIHW) 'standard analysis' flag was absent, that is, care type was newborn with unqualified days only (7.3), organ procurement – posthumous (9), or hospital boarder (10).
- Injuries due to *Complications of surgical and medical care (T80 – T88)* and *Sequelae of injuries, of poisoning and of other consequences of external causes (T90 – T98)* are excluded.

Estimating index cases, not counting separations

Each record in the NHMD refers to a single episode of care in a hospital. Some injury incidents result in more than one episode of care and, therefore, more than one record.

To minimise the impact of overcounting where a person experienced multiple episodes of care relating to the same condition, the following criteria are applied to estimate incidents:

- Excludes records where admission mode is *transfer from another hospital* (1).
- Excludes records where admission mode is *statistical admission* (2) and care type is not acute (1, 7.1, 7.2).
- Excluding records where *care involving use of rehabilitation procedures* (Z50) appears as an additional diagnosis and care type is not acute (1, 7.1, 7.2).

Injury classifications from ICD-10-AM

Diagnosis, intervention, activity, place of occurrence and external cause data for 2022–23 were reported to the NHMD using classifications from the 12th edition of *the International statistical classification of diseases and related health problems, 10th revision, Australian modification* (ICD-10-AM) (IHPA 2022), incorporating the *Australian classification of health interventions* (ACHI).

In tables and figures, information on diagnoses, external causes, activity, place of occurrence and interventions are presented using the codes and abbreviated descriptions of the ICD-10-AM/ACHI. Full descriptions of the categories are available in ICD-10-AM/ACHI publications on the Independent Health and Aged Care Pricing Authority (IHACPA) website (IHPA 2022).

Where data are presented in a time series incorporating previous reporting periods, these have been coded according to the following editions of ICD-10-AM:

- 7th edition for 2011–12 and 2012–13 hospital data
- 8th edition for 2013–14 and 2014–15 hospital data
- 9th edition for 2015–16 and 2016–17 hospital data
- 10th edition for 2017–18 and 2018–19 hospital data
- 11th edition for 2019–20, 2020-21, 2021–22 hospital data
- 12th edition for 2022–23, 2023-24, 2024–25 hospital data.

Categorising external causes of injury

The NHMD is structured so that the first listed external cause for a record relates to the first listed injury diagnosis (principal diagnosis). While multiple external causes may be recorded for a separation, we report only one cause for each injury, referred to as 'nominal external cause' in these notes. The following steps are followed to determine the nominal external cause for each injury hospitalisation:

1. The first reported external cause is taken to be the nominal external cause.
2. If the nominal external cause, as determined by step 1, is U90.0 (*Healthcare associated Staphylococcus aureus bacteraemia*) or a supplementary factor (Y90–Y98), then the second reported code is taken to be the nominal external cause.
3. If the nominal external cause, after steps 1 and 2, relates to complications of medical and surgical care (Y40–Y84), sequelae of external causes of morbidity and mortality (Y85–Y89), or a supplementary factor code (Y90–Y98), then the record is excluded.

The categorisation of external causes using ICD-10-AM codes are detailed in Appendix tables to technical notes for Injury in Australia [XLSX 150kB].

Categorising type, body part, activity and place of injury

Type of injury and body part injured are based on the patient's principal diagnosis. Principal diagnosis is the diagnosis chiefly responsible for occasioning the episode of care for the patient as defined by ICD-10-AM codes. The principal diagnosis details the type of injury sustained such as fractures, dislocations, nerve injuries and burns, and the body part injured such as head, neck, ankle and foot.

To categorise injuries by type and body part injured, Injury in Australia's principal diagnosis matrix has been applied (as outlined in the Appendix tables to technical notes for Injury in Australia [XLSX 150kB]).

The sum of injuries by body part may not equal the total number of hospitalised injury cases because some injuries are not described in terms of body region.

Activity while injured and *place of occurrence of injury* are reported as recorded from external cause of injury data. Where appropriate these may be clustered into like categories and presented as such.

Analysis methods

The Australian ERP as at 30 June 2001 is used as the standardising population throughout the report. Age-standardisation of rates enables valid comparison across years and/or jurisdictions without being affected by differences in age distributions.

Population-based rates of injury tend to have similar values from one year to the next. Exceptions to this can occur (for example, due to a mass-casualty disaster), but are unusual in Australian injury data. Some year-on-year variation and short-run fluctuations are to be expected, so small changes in a rate over a short period do not provide a firm basis for asserting that a trend is present.

All rate calculations (Table T3) utilise a denominator based on the estimated resident population (ERP) calculated as at the midpoint of each financial year. For example, for the reporting period 2024–25, the denominator population is the June 2024 ERP + the June 2025 ERP, divided by 2. This is used as the denominator for age specific/crude and age standardised rates. Rates are calculated for each financial year unless otherwise noted.

Table T3: Calculation methods for measures of injury hospitalisations in Australia

Measure	Numerator	Denominator	Calculation
Population (used for rates)	June 2024 population + June 2025 population	2	Numerator ÷ Denominator
Crude or age-specific rate of hospitalisation	Number of cases of injury hospitalisation per defined category (e.g. age group)	Estimated Australian population as at mid-point of financial year	(Numerator ÷ Denominator) x 100,000
Age-standardised rate (ASR). ASRs were derived using 5-year age groups up to 85+. ASRs for First Nation populations were derived using 5-year age groups up to 65+.	Expected events per age group in standard population = crude rate of hospitalisation x standard population (for each corresponding age group)	n.a.	The direct method of standardisation is used. (Sum of numerators across all age groups ÷ total standard population) x 100,000
Average length of stay	Number of patient bed days	Number of cases	Numerator ÷ Denominator
Change in rates	n.a.	n.a.	Estimated trends in age-standardised rates were reported as average annual percentage changes.

n.a. Not Applicable

Note that 'average length of stay', as presented in this report, does not include some patient days potentially attributable to injury. It does not include days for most aspects of injury rehabilitation, which cannot be reliably assigned without information enabling identification of all admitted episodes associated with an injury case.

Timeseries

10-year time series rates may not match historically published rates due to changes in denominator data over time, jurisdictional updates in numerator data or retrospective refreshing of numerator or denominator data across different data sources.

Presentation of data

Persons totals include cases for which sex was not reported.

All age totals include cases where age was not reported.

Percentages, rates (crude/age-specific and age-standardised), and average length of stay (ALOS) figures are rounded to 1 decimal place. Percentages may not add up to 100.0 because of rounding. Both crude/age-specific rates and age-standardised rates are calculated per 100,000 population.

Data suppression and confidentiality

Aggregated injury hospitalisations data are usually presented in tables, graphs, or maps. To avoid attribute disclosure and minimise risk of potentially re-identifying a person, data suppression rules have been applied.

Consequential suppression may also be applied to prevent a suppressed cell from being calculated. This is often done by suppressing table cells in the same row or column or suppressing the table totals.

Data may be suppressed to maintain the privacy or confidentiality of a person, or because a proportion or other measure is related to a small number of events and may therefore not be reliable. Data may also be suppressed to avoid attribute disclosure. The abbreviation 'n.p.' (not published) has been used in tables to denote these suppressions. The suppressed information remains in the totals.

Counts

- Counts less than 5 are suppressed and consequential suppression is applied.
- When data are disaggregated by geography location, counts for areas where the population is less than 1,000 are suppressed.

Crude rates

- Crude rates with counts (numerator for calculation) less than 10 are suppressed.
- If the corresponding counts measure is suppressed, the crude rate has been suppressed.
- When data are disaggregated by geography location, counts for areas where the population is less than 100 are suppressed.

Age-standardised rates

- Age-standardised rates with counts (numerator for calculation) less than 20 are suppressed.
- If the corresponding counts measure is suppressed, the age-standardised rate has been suppressed.
- When data are disaggregated by geography location, counts for areas where the population is less than 30 are suppressed.

Z-score

No suppression applied.

Data quality

A summary of data notes and data quality issues for the NHMD can be found in the Admitted Patient Care technical notes and appendices on Hospitals. Additionally, please refer to the interactive timeline in Figure 1 of these technical notes for key data quality related changes over time.

The key issues are:

Indigenous status

The AIHW report *Indigenous identification in hospital separations data: quality report* (AIHW 2013) presents findings on the quality of First Nations identification in hospital separations data in Australia, based on studies conducted in public hospitals during 2011. Private hospitals were not included in the assessment. The findings indicated that all jurisdictions have sufficient quality of Indigenous identification for reporting from 2010–11 onwards. An estimated 88% of First Nations patients were correctly identified in Australian public hospital admission records in 2011–12 (AIHW 2013). Across states and territories, this proportion ranged from 58% (confidence interval, 46-69%) in the Australian Capital Territory to 98% (96-99%) in the Northern Territory over the same time period.

Variation in state and territory coding practices

The emergency department admission policy was changed for New South Wales (NSW) hospitals in 2017–18. Episodes of care delivered entirely within a designated emergency department or urgent care centre are no longer categorised as an admission regardless of the amount of time spent in the hospital. This narrowing of the categorisation has had the effect of reducing the number of admissions recorded in NSW from the 2017–18 financial year. For NSW, the effect was a significant decrease (3.7%) in all public hospital admissions in 2017–18 compared to 2016–17. The impact of the change was felt disproportionately among hospitalisations for injury and poisoning. According to NSW Health, the number of hospitalisations for injury and poisoning in NSW decreased by 7.6% between 2016–17 and 2017–18, compared to a usual yearly increase of 2.8% (Centre for Epidemiology and Evidence 2019).

The change in NSW's emergency department admission policy may have had different effects on case numbers within different external cause categories. This is because different types of injury have a different likelihood of requiring prolonged care in an emergency department, but without an admission to a hospital ward.

Due to the size of the contribution of NSW data to the national total, Australian data from 2017–18 should therefore not be compared with data from previous years.

Cross-border flow of patients

Data on state or territory of hospitalisation should be interpreted with caution because of cross-border flows of patients. This is particularly the case for the Australian Capital Territory. In 2022–23, 17% of separations for Australian Capital Territory hospitals were for patients who lived in New South Wales.

References

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IHPA (Independent Hospital Pricing Authority) (2022). Australian Classification of Health Interventions (ACHI). Sydney: IHPA.

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Injury deaths

The underlying cause of death (UCoD) code represents the disease or injury that initiated the train of morbid events leading to a person's death, according to information available to the coder. If a death was due to an injury, the ICD-10 requires that the external cause be entered as the UCoD.

Multiple causes of death (MCoD) codes represent all the morbid conditions, diseases and injuries which are listed on the death certificate. They include all the factors in the morbid train of events leading to death: the underlying cause, the immediate cause, any intervening causes, and any conditions that contributed. This is especially helpful for chronic conditions, which often involve more than one illness.

Coding is according to the ICD-10 (WHO 2019), which includes a chapter for injuries and another for external causes.

Inclusion criteria

A death due to injury is defined by 2 criteria:

1. The UCoD was an external cause code in the range V01–Y36.
2. At least one MCoD was an external cause in the range V01–X59 or Y10–Y34, and at least one other was a code for an injury (S00–T75 or T79). MCoD is not considered for records where the UCoD indicates self-harm or assault.

The sum of deaths by cause may be greater than the total number of injury deaths because some deaths are due to multiple causes.

Deaths data are commonly recorded according to the calendar year in which the death was registered. However, in this report data are presented according to the financial year in which each death occurred.

Categorising causes of death

The code range V01–Y36 includes all unintentional (accidental) deaths, intentional self-harm (suicide), homicides, and deaths where intent remained undetermined. These codes provide information around the circumstances of the death, such as details of a transport accident, drowning, asphyxiation, effects of radiation, heat, pressure, deprivation, and maltreatment.

The code range S00–T75 and T79 includes traumatic injuries (such as fractures and lacerations), burns, poisoning and toxic effects of substances. The codes also provide information about the single, multiple, or unspecified body regions affected such as head, shoulder, knee and foot.

Table T4: Categories of external cause of injury death, and associated cause-specific criteria used for cause of death categorisation

External cause	Cause-specific criteria
Transport	Records that included the following ICD-10 codes were included: • the UCoD was *Transport accident* (V01–V99); or • the MCoD included codes for *Transport accident* (V01–V99) and for *Injury* (S00–T75 or T79). Suicide and homicide deaths (UCoD X60–Y09) were excluded.
Drowning	Records that included the following ICD-10 codes were included: • the UCoD was *Accidental drowning and submersion* (W65–W74); or • the MCoDs included codes for *Accidental drowning and submersion* (W65–W74) and for Injury (S00–T75 or T79); or • the MCoDs included codes for *Drowning and non-fatal submersion* (T75.1) and for an *Unintentional* external cause of injury (V01–X59). Suicide and homicide deaths (UCoD X60–Y09) were excluded.
Choking and suffocation	Records that included the following ICD-10 codes were included: • the UCoD was *Other accidental threats to breathing* (W75–W84); or • the MCoDs included codes for *Other accidental threats to breathing* (W75–W84) and for Injury (S00–T75 or T79). Suicide and homicide deaths (UCoD X60–Y09) were excluded.
Accidental poisoning	Records that included the following ICD-10 codes were included: • the UCoD was *Unintentional poisoning by pharmaceuticals* (X40–X44) and *Unintentional poisoning by substances other than pharmaceuticals* (X45–X49); or • the MCoDs included codes for *Unintentional poisoning by pharmaceuticals* (X40–X44) and *Unintentional poisoning by substances other than pharmaceuticals* (X45–X49) and for Injury (S00–T75 or T79); or • the MCoDs included codes for T*oxic effects of pharmaceuticals* (T36–T50) and *Toxic effects of substances other than pharmaceuticals* (T51–T65) and for *Unintentional external cause of injury* (V01–X59). Suicide and homicide deaths (UCoD X60–Y09) were excluded. According to ICD-10 coding practices, accidental poisoning deaths involving multiple drugs, medicaments or biological substances are categorised as X44 (*Accidental poisoning by and exposure to other and unspecified drugs, medicaments and biological substances*). This is specific to deaths data, as hospitalisation records follow ICD-10-AM, with different coding rules.

Falls	Records that included the following ICD-10 codes were included: • the UCoD was *Unintentional fall* (W00–W19); or • the MCoDs included codes for *Unintentional fall* (W00–W19) and for Injury (S00–T75 or T79); or • the code for *Exposure to unspecified factor* (X59) appeared anywhere on the record (either as a UCoD or MCoD) and the MCoDs included a code for *Fracture*. Suicide and homicide deaths (UCoD X60–Y09) were excluded. The codes for fractures are S02, S12, S22, S32, S42, S52, S62, S72, S82, S92, T02, T08, T10, T12, and T14.2.
Thermal causes	Records that included the following ICD-10 codes were included: • the UCoD was *Exposure to smoke, fire and flames* (X00–X09) or *Contact with heat and hot substances* (X10–X19); or • the MCoDs included codes for *Exposure to smoke, fire and flames* or *Contact with heat and hot substances* (X00–X19) and for Injury (S00–T75 or T79); or • the MCoDs included codes for *Burns* (T20–T31) and for *External causes of unintentional injury* (V01–X59). Suicide and homicide deaths (UCoD X60–Y09) were excluded.
Contact with objects	Records that included the following ICD-10 codes were included: • the UCoD was *Exposure to inanimate mechanical forces* (W20–W49); or • the MCoDs included codes for *Exposure to inanimate mechanical forces* (W20–W49) and for Injury (S00–T75 or T79). Suicide and homicide deaths (UCoD X60–Y09) were excluded.
Electricity and air pressure	Records that included the following ICD-10 codes were included: • the UCoD was *Exposure to electric current, radiation and extreme ambient air temperature and pressure* (W85–W99); or • the MCoDs included codes for *Exposure to electric current, radiation and extreme ambient air temperature and pressure* (W85–W99) and for Injury (S00–T75 or T79). Suicide and homicide deaths (UCoD X60–Y09) were excluded.
Contact with living things	Records that included the following ICD-10 codes were included: • the UCoD was *Exposure to animate mechanical forces* (W50–W64) or *Contact with venomous animals and plants* (X20–X29) or *Allergy to animals* (Y37.6); or • the MCoDs included codes *Exposure to animate mechanical forces* (W50–W64) or *Contact with venomous animals and plants* (X20–X29) or *Allergy to animals* (Y37.6), and a code for Injury (S00–T75 or T79). Suicide and homicide deaths (UCoD X60–Y09) were excluded.
Exposure to forces of nature	Records that included the following ICD-10 codes were included: • the UCoD was *Exposure to forces of nature* (X30–X39); or • the MCoDs included codes *for Exposure to forces of nature* (X30–X39) and for Injury (S00–T75 or T79). Suicide and homicide deaths (UCoD X60–Y09) were excluded.
Overexertion, travel and privation	Records that included the following ICD-10 codes were included: • the UCoD was *Overexertion, travel and privation* (X50–X57); or • the MCoDs included codes for *Overexertion, travel and privation* (X50–X57) and for Injury (S00–T75 or T79). Suicide and homicide deaths (UCoD X60–Y09) were excluded.
Suicide	Records that included the following ICD-10 codes were included: • the UCoD was *Intentional self-harm* (X60–X84).
Homicide	Records that included the following ICD 10 codes were included: • the UCoD was *Assault* (X85–Y09) or *Legal intervention and operations of war* (Y35–Y36).
Event of undetermined intent	Records that included the following ICD-10 codes were included: • the UCoD was *Event of undetermined intent* (Y10–Y34); or • the MCoDs included codes for *Event of undetermined intent* (Y10–Y34) and for Injury (S00–T75 or T79).

Analysis methods

The Australian ERP as at 30 June 2001 is used as the standardising population throughout the report. Age-standardisation of rates enables valid comparison across years and/or jurisdictions without being affected by differences in age distributions.

Population-based rates of injury tend to have similar values from one year to the next. Exceptions to this can occur (for example, due to a mass-casualty disaster), but are unusual in Australian injury data. Some year-on-year variation and short-run fluctuations are to be expected, so small changes in a rate over a short period do not provide a firm basis for asserting that a trend is present.

All rate calculations (Table T5) utilise a denominator based on the estimated resident population (ERP) calculated as at the midpoint of each financial year. For example, for the reporting period 2023–24, the denominator population is the June 2023 ERP + the June 2024 ERP, divided by 2. This is used as the denominator for age-specific/crude and age-standardised rates. Rates are calculated for each financial year unless otherwise noted.

Table T5: Calculation methods for measures of injury deaths in Australia

Measure	Numerator	Denominator	Calculation
Population (used for rates)	June 2023 population + June 2024 population	2	Numerator ÷ Denominator
Crude or age-specific mortality rate	Number of injury deaths per defined category (e.g. age group)	Estimated Australian population as at mid-point of financial year	(Numerator ÷ Denominator) x 100,000
Age-standardised mortality rate (ASR). ASRs were derived using 5-year age groups up to 85+. ASRs for First Nation populations were derived using 5-year age groups up to 65+.	Expected events per age group in standard population= crude mortality rate x standard population (for each corresponding age group)	n.a.	The direct method of standardisation is used. (Sum of numerators across all age groups ÷ total standard population) x 100,000
Change in rates	n.a.	n.a.	Estimated trends in age-standardised rates were reported as average annual percentage changes.

n.a. Not Applicable

Timeseries

10-year time series rates may not match historically published rates due to changes in denominator data over time, jurisdictional updates in numerator data or retrospective refreshing of numerator or denominator data across different data sources.

Presentation of data

The sum of the counts of death by cause may be greater than the total number of injury deaths because some cases of death have multiple causes which contributed to the death. Cases with multiple causes are counted once per cause group.

Persons totals include deaths for which sex was not reported.

All age totals include deaths where age was not reported.

Crude/age-specific rates and age-standardised rates are calculated per 100,000 population and are rounded to one decimal place (for example, 3.4 per 100,000).

Data suppression and confidentiality

Data may be suppressed to maintain the privacy or confidentiality of a person, or because a proportion or other measure is related to a small number of events and may therefore not be reliable. Data may also be suppressed to avoid attribute disclosure. The abbreviation 'n.p.' (not published) has been used in tables to denote these suppressions. The suppressed information remains in the totals.

Counts

- Counts of 1and 2 are suppressed and consequential suppression is applied.
- First Nations counts of 1 and 2 are suppressed and consequential suppression is applied.
- Combined 0–14 age group for the suicide category.

Crude rates

- Crude rates with counts (numerator for calculation) less than 10 are suppressed.
- If the corresponding counts measure is suppressed, the crude rate has been suppressed.

Age-standardised rates

- Age-standardised rates with counts (numerator for calculation) less than 20 are suppressed.
- If the corresponding counts measure is suppressed, the age-standardised rate has been suppressed.

Z-score

No suppression applied.

Data quality

The data quality statements underpinning the AIHW National Mortality Database can be found in the following Australian Bureau of Statistics (ABS) publications:

- ABS quality declaration summary for Deaths, Australia methodology
- ABS quality declaration summary for Causes of Death, Australia methodology
- For more information on the AIHW National Mortality Database see Deaths data at AIHW.

Deaths registered in 2021 and earlier are based on the final version of cause of death data; deaths registered in 2022 are based on the revised version; and deaths registered in 2023 and 2024 are based on the preliminary version. Revised and preliminary versions are subject to further revision by the Australian Bureau of Statistics (ABS).

Please also refer to the interactive timeline for details of changes in deaths data over time.

Errors in deaths data

The data presented in this report are subject to 2 types of statistical error, non-random and random (a third type of statistical error, sampling error, does not apply in this report, because none of the data sources used involved probability sampling).

Non-random error

Some level of non-random error is to be expected in administrative data collections, such as the NMD on which this report relies. For example, non-random error could occur if the approach to assigning cause codes to deaths were to differ systematically between jurisdictions, or over time. While systems are in place to encourage uniform data collection, and coding and scrutiny of data during analysis include checking for patterns that might reflect non-random error, some error remains.

Random error

The values presented in the report are subject to random error, or variation. Variation is relatively large when the case count is small (especially if less than about 10). Variation is small enough to be mostly unimportant when the case count is larger (that is, more than a few tens of cases).

Some of the topics for which results are reported compare groups that vary widely in case count, largely due to differences in population size (for example, the population of New South Wales is more than 30 times as large as the Northern Territory population, and the population of *Major cities* is nearly 90 times that of *Very remote* areas). In this situation, year-to-year changes in counts or rates for the smaller-population groups might be subject to large random variation. Such fluctuations could potentially be misinterpreted as meaningful rises or falls.

References

WHO (World Health Organization) (2019) ICD-10: international statistical classification of diseases and related health problems: tenth revision, WHO, Geneva.

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Notes

Scope of this report

This report presents information on 1,829,829 emergency department (ED) presentations and 584,539 hospitalisations with a primary diagnosis of injury between 1 July 2024 and 30 June 2025. Data on 15,765 deaths is presented where injuries are recorded in finalised cause of death records from 1 July 2023 to 30 June 2024.

This report updates information about injuries overall annually. Each specific cause of injury will be separately updated throughout the year.

Trends over the preceding decade of injury hospitalisations between 1 July 2015 and 30 June 2025, and injury deaths between 1 July 2014 and 20 June 2024 are included in this report.

Please refer to the technical notes for further details on methodology, including inclusion and exclusion criteria for counting injuries in ED, hospital and deaths data sources.

Amendments

20 November 2025 – A delay in receiving clearance from all Australian jurisdictions for the AIHW to release data on injury emergency department presentations at the level of state and territory resulted in removal of affected data tables prior to the embargo date. Clearance was subsequently received during the embargo period, on 17 November 2025.

14 July 2025 – On the Data page, State level data tables have been made available once more.

Acknowledgements

Injury in Australia was prepared by staff in the Injury Data Unit at the AIHW.

The project was funded by the Australian Government Department of Health, Disability and Ageing.

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Data

Data tables: Falls injury hospitalisations and deaths, Australia

Data

Injury in Australia data tables - Falls Tables 1 to 13
XLSX 467kB

Data tables: Injury ED presentations, hospitalisations and deaths, Australia

Data

Data tables – Injury ED presentations (Tables E1-E19) and hospitalisations (Tables H1-H24) up to 2024–25 – Injury deaths (Tables D1-D13) up to 2023–24
XLSX 649kB

Data tables: Accidental poisoning injury hospitalisations and deaths, Australia

Data

Injury in Australia data tables - Poisoning Tables 1 to 8
XLSX 195kB

Data tables: Assault injury hospitalisations and homicide deaths, Australia

Data

Injury in Australia data tables - Assault Tables 1 to 8
XLSX 416kB

Data tables: Objects-related injury hospitalisations and deaths, Australia

Data

Injury in Australia data tables - Objects Tables 1 to 9
XLSX 585kB

Data tables: Transport injury hospitalisations and deaths, Australia

Data

Injury in Australia data tables - Transport Tables 1 to 10
XLSX 230kB

Data tables: Injury ED presentations, hospitalisations and deaths data, by State and Territory

Data

Injury in Australia data tables - ED presentations (2023-24), hospitalisations (2023-24) and deaths (2022-23) by State and Territory (all causes of injury)
XLSX 513kB

Data tables: Injury in Australia (Machine Readable)

Data

Injury in Australia data tables in machine readable format: Tables E1-E19, H1-H24, D1-D13, Falls 1-13, Objects 1-9, Transport 1-10, Poisoning 1-9, Assault 1-8
XLSX 7.2MB

Data tables: Injury in Australia (Machine Readable) by State and Territory

Data

Injury in Australia data tables in Machine Readable format, including all Injury in Australia hospitalisations and deaths tables by State and Territory
XLSX 1.4MB

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For the most recent data tables, visit the Data page.

Data tables from earlier versions of the Injury in Australia are available for download on this page.

Injury in Australia - Data tables (2025 edition)

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Data tables (2025 edition)

For the most recent data tables, visit the Data page.

Data tables accompanying the 2025 edition of Injury in Australia are available for download on this page. They contain data for the following years:

- hospitals data (ED presentations and hospitalisations) up to 2023–24
- mortality data up to 2022–23

(Archived) Data tables: Injury ED presentations, hospitalisations and deaths, Australia

Data | *25 Nov 2025*

Archived Injury in Australia data tables – Injury ED presentations (Tables E1-E19) and hospitalisations (Tables H1-H29) up to 2023–24 – Injury deaths (Tables D1-D22) up to 2022–23
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Data | *25 Nov 2025*

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